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THE DOCTOR IS (NOT) IN: IMPLICATIONS OF INCREASED MEDICAL MALPRACTICE LITIGATION ON TEAM PHYSICIANS IN THE NATIONAL FOOTBALL LEAGUE

I. It Only Takes One Person, One Patient, One Doctor, One Moment to Change Your Life Forever: Introduction to Medical Malpractice Litigation in the National Football League and Its Profound Impact on Team Physicians

"While I live in constant pain and will never get back on the field, I hope this decision sends a message to teams' medical staffs that players are people, not just contracts," stated former Philadelphia Eagles captain Chris Maragos.[1] In February 2023, Maragos was awarded $43.5 million in a medical malpractice lawsuit against orthopedic surgeon Dr. James Bradley and Rothman Orthopaedic Institute.[2] This lawsuit stemmed from a knee injury that Maragos suffered during a game against the Carolina Panthers on October 12, 2017.[3] Both Dr. Bradley and Rothman Orthopaedics acted as the Philadelphia Eagles' team physicians and oversaw Maragos' rehabilitation after his knee injury.[4] A Philadelphia County jury found that both Dr. Bradley's and Rothman Orthopaedics's negligent treatment resulted in "further complications and the premature end of

1. *See* Matias Grez, *Former Philadelphia Eagles Captain Chris Maragos Awarded $43.5 Million in Medical Malpractice Case*, CNN (Feb. 15, 2023, 11:00 AM), https://www.cnn.com/2023/02/15/sport/chris-maragos-wins-medical-malpractice-case-spt-intl/index.html [https://perma.cc/B623-SUZ7] (summarizing Maragos' thoughts towards his career ending injury after Philadelphia County jury found in his favor in medical malpractice lawsuit); *see also* Tom Ignudo, *Ex-Eagle Chris Maragos Awarded Millions in Injury Case*, CBS News (Feb. 13, 2023, 7:01 PM), https://www.cbsnews.com/philadelphia/news/chris-maragos-eagles-civil-case-knee-injury-super-bowl-nfl/ [https://perma.cc/QG7A-B5BT] ("On Sunday, my team played in the Super Bowl, and I could only watch and wonder whether I could have been out there with them had I received proper medical care.").

2. *See* Grez, *supra* note 1 (explaining that Bradley was found to be 67% negligent with judgment of $29.2 million and Rothman Orthopaedics was found to be 33% negligent with judgment of $14.3 million).

3. *See id.* (indicating Maragos was diagnosed with torn right knee posterior cruciate ligament and received surgery that resulted in improper rehabilitation, effectively ending his NFL career).

4. *See* John McMullen, *Chris Maragos Alleges Former Medical Team Cut Short His NFL Career*, Sports Illustrated (Feb. 2, 2023, 4:19 PM), https://www.si.com/nfl/eagles/news/chris-maragos-alleges-former-medical-team-cut-short-his-nfl-career [https://perma.cc/HDR3-Q7UR] (explaining that Maragos alleged Dr. Bradley failed to address his torn meniscus, which caused further damage to his knee).

Maragos' National Football League (NFL) career."[5] Since Maragos' verdict, Rothman Orthopaedic Institute has ended their relationship with the Philadelphia Eagles due to increased potential for subsequent liability.[6]

Maragos is not the only NFL player who has brought a successful medical malpractice lawsuit in the last decade.[7] In 2014, former Baltimore Ravens cornerback, Samari Rolle, was awarded $650,000 after a jury found that improper post-surgical care ended Rolle's NFL career.[8] In 2022, former New York Giants running back, Michael Cox, was awarded $28.5 million after a failed ankle surgery ended his NFL career.[9] The publicity of these injuries show that there are genuine concerns from both NFL players and the general public regarding the quality of post-injury care that professional athletes receive.[10]

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5. *See* Ignudo, *supra* note 1 (explaining that MRI revealed Maragos' knee injury worsened while undergoing rehabilitative care with Dr. Bradley and Rothman Orthopaedics).

6. *See* John McMullen, *Rothman Orthopaedic Ends Relationship with Eagles over Chris Maragos Damages*, SPORTS ILLUSTRATED (June 21, 2024), https://www.si.com/nfl/eagles/news/rothman-orthopaedic-ends-relationship-with-eagles-over-chris-maragos-damages-01j0vj8n2r1h [https://perma.cc/V7YK-2DEJ] (stating that Rothman Orthopaedics believed risk of potential future liability resulting from continued partnership with Philadelphia Eagles was too high to continue providing care to players and reaffirmed commitment to providing care to other patients).

7. *See* Christopher T. Nace, *Risking Life for Limelight: Medical Malpractice in the NFL*, LEGAL EXAM'R (Feb. 9, 2023), https://washingtondc.legalexaminer.com/health/risking-life-for-limelight-medical-malpractice-in-the-nfl/ [https://perma.cc/LCR5-DC3D] (discussing four examples of medical malpractice cases in NFL during last decade).

8. *See id.* (explaining factual basis of Rolle's medical malpractice lawsuit filed against spinal surgeon, Dr. Craig Brigham).

9. *See* Steven Taranto, *Former Giants Running Back Michael Cox Wins over $28 Million in Medical Malpractice Case*, CBS SPORTS (Sept. 24, 2022, 6:18 PM), https://www.cbssports.com/nfl/news/former-giants-running-back-michael-cox-wins-over-28-million-in-medical-malpractice-case/ [https://perma.cc/L5SQ-8ADR] (explaining that Cox sued Dr. Dean Lorich and New York Presbyterian Hospital after botched ankle surgery permanently ended his NFL career). The jury awarded Cox "$15.5 million for future pain and suffering, $12 million in lost earnings, and $1 million in past pain and suffering." *See id.* (discussing outcome of Cox's suit against Dr. Lorich and New York Presbyterian Hospital).

10. *See* Brad Mielke & Ivan Pereira, *NFL Under Fire over Its Concussion Protocol Following Tua Tagovailoa's Back-to-Back Game Hits*, ABC NEWS (Oct. 3, 2022, 6:54 PM), https://abcnews.go.com/Sports/nfl-fire-concussion-protocol-tua-tagovailoas-back-back/story?id=90918691 [https://perma.cc/7QLK-U7SP] (acknowledging frustrations and criticisms voiced by former NFL players and football fans when Dolphins' medical team allowed Tagovailoa to return to play without conducting concussion evaluation); *see also* Sam Eifling, *Walk it Off, Champ: Why NFL Team Doctors Are Ethically Compromised*, SLATE (Jan. 30, 2013, 4:52 PM), https://slate.com/culture/2013/01/nfl-team-doctors-the-problem-with-pro-footballs-medical-sponsorship-deals.html [https://perma.cc/TZT4-SDY2] (discussing conflicts of interest that arise due to current employment model of team physicians). The current employment model for team physicians creates a conflict of interest because the doctors are

In response to the increasing number of medical malpractice lawsuits filed by professional athletes, the American Orthopaedic Society for Sports Medicine (AOSSM) published an open letter cautioning the public about the negative effects that an influx of successful lawsuits would have on team physicians.[11] In the open letter, AOSSM stated "recent and ongoing litigation may have an enormous negative impact on the medical care of competitive and elite athletes."[12] The predicted negative impact stems from the concern that increased costs from lawsuit verdicts and settlements will cause liability insurance costs to skyrocket, decreasing the number of qualified physicians that would want to work in this specialized field.[13] AOSSM argues that to minimize the increasing collateral consequences of litigation on team physicians, changes must be made to the NFL's concept of a team physician.[14] One change,

employed by the NFL teams, not the players they treat. *See id.* (explaining conflict of interest that is exacerbated by NFL team physician's current employment model).

11. *See* Press Release, Am. Orthopaedic Soc'y for Sports Med., Protecting Athletes & Team Physicians Who Care for Them Amid Rising Complexity & Liability Risk (June 12, 2023) [hereinafter Am. Orthopaedic Soc'y Press Release], https://www.sportsmed.org/about-us/news/protecting-athletes-and-team-physicians-who-care-for-them-amid-rising-complexity-and-liability-risk [https://perma.cc/RP6A-LK7K] (arguing higher injury risk of athletes threatens relationship between team physicians because this risk contributes to elevated liability for physicians and significant damages awards). This relationship is threatened because of the increased medical malpractice litigation against team physicians, less doctors will want to become team physicians. *See id.* (emphasizing physicians must work together to fight against damage increased medical malpractice litigation will do to profession); *see also AOSSM Pens Open Letter Demanding Physician Testimonies in Quality of Care Litigation*, HEALIO (June 16, 2023), https://www.healio.com/news/orthopedics/20230616/aossm-pens-open-letter-demanding-physician-testimonies-in-quality-of-care-litigation [https://perma.cc/JFN8-JNRF] (explaining that open letter received over twenty-seven different signatures from supporting organizations, including MLB Team Physician Association and NBA Physician Association).

12. *See* Am. Orthopaedic Soc'y Press Release, *supra* note 11 (discussing recent influx of medical malpractice lawsuits against sports-medicine practitioners and heightened legal risks and quality of care issues that now result from practicing sports medicine).

13. *See* Stephanie Bell, *Medical Groups: Liability May Limit Care for Top Athletes*, ESPN (June 12, 2023, 3:23 PM), https://www.espn.com/espn/story/_/id/37841581/medical-groups-alarmed-rising-liability-costs [https://perma.cc/PHJ3-C666] (arguing risk of heightened liability for sports-medicine practitioners may deter young doctors from sports medicine field and negatively impact ability of teams to find and employ team physicians). Obtaining malpractice coverage through insurance companies could become nearly impossible for younger physicians as insurance premiums increase. *See id.* (discussing increased hurdles to obtaining malpractice insurance for team physicians). Some insurance companies have indicated that they will not cover qualified physicians if they provide care for professional athletes. *See id.* ("A prominent orthopedic surgeon who takes care of professional athletes has already indicated that his insurance will no longer allow him to take care of this population because of this very issue.").

14. *See* Am. Orthopaedic Soc'y Press Release, *supra* note 11 (arguing that current team physician model will result in "unnecessarily high legal risk" due to higher injury risks of athletes resulting in decreased earning potential).

suggested by AOSSM, would promote a transparent relationship between the patient-players, the team physician, and the public, to ensure everyone properly understands the team physician's role and responsibilities.[15]

The ultimate question is: where does one draw the line between vigorously protecting the health and safety of professional athletes and limiting the liability of team physicians?[16] Team physicians play a critical role in ensuring athletes play at their best level, and as a result, have developed a special relationship with their players.[17] The potential impact of increased liability for these physicians jeopardizes this relationship and their role as we know it.[18] However, team physicians still need to take appropriate responsibility for the consequences of their actions if they breach the standard of care when treating a professional athlete.[19] Therefore, team physicians need to be acutely aware of conflicting interests between their employment with the team and their obligations to provide care to the players.[20]

This Comment considers the implications of increased medical malpractice litigation by NFL athletes against their team physicians, and how the Collective Bargaining Agreement (CBA) should be modified to ensure adequate protections for both team physicians and NFL players.[21] This Comment also provides solutions to

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15. *See id.* (noting effort needs to be made to preserve role of team physician, which could require new expert testimony standard). There needs to be a "concerted effort" to preserve the future of sports medicine. *See id.* (arguing role of team physician has "undergone significant evolution with regard to advancement of required medical training and expertise"); *see also* Bell, *supra* note 13 (arguing recent medical malpractice indicates higher liability concerns, which could affect availability of qualified team physicians to treat professional athletes).

16. *See* Bell, *supra* note 13 (arguing that quality and availability of care for top athletes may drastically decrease if steps are not taken to limit liability of team physicians).

17. *See* Am. Orthopaedic Soc'y Press Release, *supra* note 11 (implying that team physicians develop special relationship with professional athletes that they treat). This idea follows that both team physicians and professional athletes share goals in promoting the athletes' wellbeing so they can continue playing. *See id.* (detailing mutual goals of athletes and team physicians).

18. *See* Mark D. Miller, *Presidential Address of the American Orthopaedic Society for Sports Med.: AOSSM LI*, 51 AM. J. OF SPORTS MED. 3634, 3636 (2023) (acknowledging that some team physicians have been "subjected to excessive malpractice claims and unqualified expert testimony that threaten their role and our Society as a whole").

19. *See* Matthew J. Mitten, *Team Physicians as Co-Employees: A Prescription That Deprives Professional Athletes of an Adequate Remedy for Sports Medicine Malpractice*, 50 ST. LOUIS U. L. J. 211, 216 (2005) (arguing that team physicians should still be held "accountable if their malpractice causes harm to players whose health and safety are entrusted to them").

20. For further discussion of whether the collective bargaining agreement should be modified to protect both team physicians and players, see *infra* notes 224–244 and accompanying text.

21. For further discussion of the implications of increasing liability on team physicians, see *infra* notes 171–175 and accompanying text.

minimize the effects of medical malpractice litigation on the team physician.[22] Section II details the role of the team physician in the NFL, the standard of care used and evaluated in medical malpractice litigation involving team physicians, and explores differences in the role of the team physician in other professional sports leagues, such as the National Basketball Association (NBA), Major League Baseball (MLB), and the National Hockey League (NHL).[23] Section III analyzes the problems surrounding heightened liability of team physicians and contemplates how these issues can be rectified by modifying the CBA, or changing their employment model.[24] Section III also provides potential solutions to minimize the increasingly negative effects of medical malpractice litigation on team physicians.[25] Lastly, Section IV summarizes the need for changes in the role of team physicians to preserve the unique relationship they have with NFL players.[26]

II. THIS IS YOUR STARTING LINE: THE ROLE OF TEAM PHYSICIANS IN THE NFL, STANDARDS OF CARE, AND TEAM PHYSICIANS IN OTHER PROFESSIONAL SPORTS LEAGUES

A. Role of the Team Physician in the NFL and the Standard of Care

What exactly is a "team physician?"[27] While most people consider sports medicine to be a contemporary practice, sports medicine

22. For further discussion of expert testimony solutions, see *infra* notes 176–196 and accompanying text.

23. For further discussion of team physicians' training and standard of care, see *infra* notes 37–88 and accompanying text.

24. For further discussion of the implications of increasing liability on team physicians, see *infra* notes 171–255 and accompanying text.

25. For further discussion of expert testimony solutions, see *infra* notes 176–196 and accompanying text.

26. For further discussion of the implications of increasing liability on team physicians, see *infra* notes 171–255 and accompanying text.

27. *See* Am. Orthopaedic Soc'y Press Release, *supra* note 11 (defining general role and responsibilities of team physicians). Team physicians dedicate their lives "to caring for athletes in unique and complex scenarios, which requires extensive time and expertise." *See id.* (emphasizing that definition and roles of team physicians are always evolving); *see also* Matthew J. Mitten, *Team Physicians and Competitive Athletes: Allocating Legal Responsibility for Athletic Injuries*, 55 U. PITT. L. REV. 129, 138 (1993) (quoting Joseph H. King, Jr, *The Duty and Standard of Care for Team Physicians*, 18 HOUS. L. REV. 657, 658 (1981)) (defining team physician as "a physician who undertakes to render professional medical services to athletic participants and whose services are either arranged for or paid for at least in part by an institution or entity other than the patient, the patient's family, or some surrogate").

dates back to Ancient Greece and Ancient Rome.[28] Many aspects of sports medicine that were present during ancient times have stood the test of time and have informed modern sports medicine practices.[29] One of these practices is the inclusion of team physicians to treat athletes so they perform their best during games.[30]

A team physician "performs professional medical services to athletes that are either arranged or paid for by an institution or an entity."[31] Aside from performing traditional medical services, team physicians today have many roles outside of practicing medicine.[32] Although medical care of athletes is their core responsibility, team physicians are also responsible for working with other medical professionals, such as medical consultants and athletic trainers.[33] Additionally, a team physician "educates athletes, coaches, parents/guardians, and administrators."[34] The intersectional responsibilities of team physicians have elevated their expectations and introduced new considerations to be made, including the need to remain

28. *See* Emin Ergen, *The Origins of Sports Medicine*, 5 ASPETAR SPORTS MED. J. 342, 342–44 (2016) (summarizing history and origins of sports medicine in Ancient Rome and Ancient Greece). The first reference to a team doctor was seen in the 2nd Century in Ancient Rome. *See id.* at 344 (acknowledging appointment of Claudius Galenos as "doctor of the gladiators").

29. *See id.* (arguing many cultures have longstanding traditions in sports medicine which has made it well-established as medical practice area).

30. *See id.* (summarizing history and importance of team physicians during major competitions). Although team physicians were used before the modern Olympic Games, these modern competitions have influenced the development of sports medicine as a field. *See id.* at 346 (emphasizing that Modern Olympic Games and creation of International Olympic Committee influenced interest in "exercise physiology and sports physiology").

31. *See* Dennis Durao, *An Endangered Species: Professional Sports Team Physicians*, 15 QUINNIPIAC HEALTH L. J. 33, 36 (2012) (quoting Twila Keim, *Physicians for Professional Sports Teams: Health Care Under the Pressure of Economic & Commercial Interests*, 9 SETON HALL J. SPORT L. 196, 199 (1999)) (arguing that definition of team physician cannot be "precisely defined due to the innumerable employment arrangements entered into between teams and their physicians").

32. *See* Stanley A. Herring, W. Ben Kibler & Margot Putukian, *Team Physician Consensus Statement*, 45 MED. & SCI. IN SPORTS & EXERCISE 1618, 1618 (2013) ("Team physicians have the leadership role in the organization, management and provision of care of athletes in individual, team, and mass participation sporting events.").

33. *See id.* at 1619 (acknowledging importance of team physicians working with other medical professionals, such as "medical consultants, certified and/or licensed athletic trainers, and other allied health care professionals").

34. *See id.* (describing intersectional role of team physicians). Besides providing medical care, team physicians are encouraged to participate in the "administrative aspects of team care." *See id.* (describing that team physicians should assist with medical management of injuries and illness, recognizing issues that could affect injury care and prevention, complying medical records and documentation, and becoming familiar with how their training can be used in connection with "athletic care network"). Specifically, it is also crucial that team physicians become familiar with their teams' emergency action plans and event preparedness. *See id.* at 1620 (illustrating wide range of administrative activities that team physician should be involved with to be successful in their role).

impartial about a player's return to play, ensuring it is medically safe despite any team pressures.[35] Moreover, the current, complex, employment arrangements of team physicians have created issues that, if left unresolved, could lead to a breakdown in trust between these physicians and their players.[36]

1. *NFL Team Physician Training, Education, and Duties to Player-Patients*

Team physicians often have specialized training and experience before being hired by the NFL.[37] Along with obtaining the Doctor of Medicine designation from medical school, team physicians are also board certified in sports medicine and often have previous experience serving as a team physician for high school or college sports teams.[38] In addition to having specialized training in sports medicine, team physicians are recommended to be proficient in the "prevention and care of musculoskeletal injuries and medical conditions encountered in sports."[39] It is also important that team physicians have knowledge of on-field medical emergency care and training using an automated external defibrillator.[40] These chosen medical professionals often provide their services to the team for free or at a discounted rate because working for a professional sports team increases their marketability, and thus their bottom line, on its

35. *See id.* at 1620–21 (describing administrative and ethical challenges of team physicians' roles).

36. For further discussion of creating change to preserve the relationship between the team physicians and NFL players, see *infra* notes 138–170 and accompanying text.

37. *See How Does a Physician Become an NFL Team Physician?*, NFL PHYSICIANS SOC'Y [hereinafter *NFL Team Physician*], https://nflps.org/faqs/ [https://perma.cc/X5YZ-MATL] (last visited Jan. 23, 2024) (acknowledging that NFL team physicians have training in areas such as orthopedic surgery, general surgery, and internal medicine and typically have previous experience being team physician for high school or college teams); *see also* Herring et al., *supra* note 32, at 1618–22 (summarizing duties of team physicians to best serve athletes in their respective roles).

38. *See* Aine Greaney, *What Does an NFL Team Physician Do?*, BD. VITALS (July 1, 2023), https://www.boardvitals.com/blog/nfl-team-physicians-sports-medicine/?srsltid=AfmBOoqsYQq3zk7O_OylsDykSck3UzMZi-RRqmU1IsQEEbzfbXEr_fry [https://perma.cc/LS2Y-WVJG] (stating all NFL team physicians starting after 2011 must be board certified in sports medicine).

39. *See* Herring et al., *supra* note 32, at 1619 (emphasizing team physicians' "special proficiency" in areas most applicable to athletes' care and wellbeing).

40. *See id.* (stating it is "essential" that team physicians know how to care for on-field medical emergencies); *see also* Stanley A. Herring, W. Ben Kibler & Margot Putukian, *Team Physician Definition, Qualifications, and Responsibilities: A Consensus Statement*, AM. ACAD. OF ORTHOPAEDIC SURGEONS 2 (June 2013), https://www.aaos.org/globalassets/about/bylaws-library/information-statements/1021-team-physician-definition-qualifications-and-responsibilities—a-consensus-statement.pdf [https://perma.cc/8YSX-QYBD] (indicating that "fundamental knowledge of on-field medical emergency care" is essential skill for team physician).

own.[41] The prestige that comes with this position, compounded with the respective NFL team's own interest in ensuring its most important players are eligible to perform, has raised concerns that a team physician may put the interest of the team above the medical care of the player-patient.[42]

The NFL CBA sets out specific requirements for its team physicians, including their education and training.[43] According to the CBA, each NFL team is required to have a board-certified orthopedic surgeon and "at least one board-certified internist, family medicine, or emergency medicine physician."[44] Additionally, it requires each of its teams to employ one or more consultants with the following certifications: neurology, cardiology, nutrition, neuropsychology, behavioral health, and pain management.[45]

The NFL CBA also outlines the duties of the team physicians providing medical care to an NFL player.[46] In addition to a team physician's duty to provide care to any NFL player, team physicians are ethically bound by their oath as physicians to provide quality care to the player-patient regardless of the fact they are retained by a specific

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41. *See* Herring et al., *supra* note 32, at 1619 (arguing that increased marketability from working with teams could risk athletes' health if team physician lets team's interests influence medical decisions they make for sake of preserving relationship). Typically, each NFL team chooses a local doctor or medical group to serve as its team physician, thus increasing the desirability of these positions in communities. *See* Mitten, *supra* note 19, at 211 (implying that these medical roles are highly sought after because of their prestigious nature and potential to increase amount of non-player patients). The increase in non-player patients is attributed to publicity of working with sports teams. *See id.* (explaining publicity of medical role in sports).

42. *See* Mitten, *supra* note 19, at 213 (describing conflict team physicians face between keeping team happy and making appropriate medical decisions for health of players).

43. *See* NFL & NFL PLAYERS' ASS'N, COLLECTIVE BARGAINING AGREEMENT 213 (Mar. 5, 2020) [hereinafter NFL & NFLPA CBA], https://nflpaweb.blob.core.windows.net/website/PDFs/CBA/March-15-2020-NFL-NFLPA-Collective-Bargaining-Agreement-Final-Executed-Copy.pdf [https://perma.cc/6UPL-PYFS] (listing requirements for team physicians' education and training, and outlining expectations and responsibilities of NFL team physicians).

44. *See id.* (explaining that team physicians must also be certified by current NFL Head Team Physician, in which team physician has been affiliated with medical staff for three years and has detailed knowledge of various NFL protocols and policies).

45. *See id.* (explaining that credentials of medical professionals must be verified by independent, third-party credentialing organization).

46. *See id.* (discussing power structure within athlete care team; team physicians may generally give their medical opinion, however, only head physicians have power to "clear" athletes for return to play after injury). A team physician has the duty to keep the head physician informed about their activities with players and may not take any action that is contrary to the medical care plan overseen by the head physician. *See id.* (explaining that only head physicians have "final authority to make, modify, or override decisions in such areas").

team.[47] If a player-patient does not believe they are receiving appropriate care from a team physician, the CBA provides all players the right to obtain a second medical opinion.[48] Additionally, the CBA provides a player-patient with the right to select a surgeon of their choice to perform any surgery so long as the team agrees to pay for the surgery.[49] Lastly, all NFL players have the right to view their medical records once during the pre-season and once after the regular season.[50]

2. *NFL CBA Injury Grievance Procedure*

The NFL CBA provides for an "Injury Grievance," which occurs when a team terminates a player's contract for not fulfilling their contractual obligations due to an injury that occurred while the player was under contract.[51] This provision requires that an Injury Grievance must be documented, in writing, within twenty-five days "from the date it became known or should have become known to the player that his contract had been terminated" because of the injury.[52] By filing an Injury Grievance, the player must follow the

47. *See id.* (acknowledging that team physicians are bound by federal, state, and local requirements, including ethical standards to player-patient). Team physicians must disclose all information relating to a player-patient's care and status to the player-patient, if the information is provided to a coach of a team. *See id.* (explaining that any information relating to player's performance or health must be provided to player-patient).

48. *See id.* at 219 (explaining team must pay all costs for player to obtain second opinion regarding "diagnosis of injury, surgical and treatment decisions, and rehabilitation and treatment protocol"); *see also What Medical Rights Do Players Have?*, NFL PLAYERS' ASS'N [hereinafter *Player Medical Rights*], https://nflpa.com/faq/what-medical-rights-do-players-have [https://perma.cc/N3PP-7747] (last visited Jan. 26, 2024) (discussing expectation that player consults with team physician prior to seeking second opinion). Additionally, a physician providing a second opinion must provide the team with a copy of the report. *See id.* (explaining relationship dynamic between independent physicians and team physicians).

49. *See* NFL & NFLPA CBA, *supra* note 43, at 219 (requiring player-patient to speak with team physician regarding surgery to receive recommendations for third-party surgeon and ensure surgeon will be paid by team).

50. *See Player Medical Rights*, *supra* note 48 (explaining that CBA allows player-patients to access medical records to evaluate their overall health level and quality of medical care). A player-patient may also request their medical record at any time during the off-season, and their personal physician may request their medical record at any time. *See id.* (acknowledging that CBA provides player-patient and their own personal physician with opportunities to view their medical records).

51. *See generally* NFL & NFLPA CBA, *supra* note 43, at 261–267 (explaining process for players to file claims once contract is terminated). Article 44 provides a formal process, including filing a written claim and answer, regarding the injury at issue. *See id.* (explaining formal hearing process after grievance is filed and clarifying that player must be evaluated by neutral physician twenty days after filing grievance).

52. *See id.* at 261 (acknowledging that after player files Injury Grievance, NFL team must file answer, in writing, to player's Injury Grievance within ten days).

mandatory procedures, including an examination by a neutral physician and a potential arbitration hearing if the case is not settled by the Grievance Settlement Committee.[53]

3. *Standard of Care in Medical Malpractice with Team Physicians*

The long list of responsibilities and complex employment model of the team physician has led to increased litigation in this area.[54] Any litigation initiated against a team physician is often for their actions in treating a player-patient, and is commonly grounded in medical malpractice claims.[55] Medical malpractice is defined as "any act or omission by a physician during treatment of a patient that deviates from accepted norms of practice in the medical community and causes an injury to the patient."[56] To state a viable claim for medical malpractice in the United States, a plaintiff must prove four elements:

> (1) the existence of a legal duty on the part of the doctor to provide care or treatment to the patient; (2) a breach of this duty by a failure of the treating doctor to adhere to the standards of the profession; (3) a causal relationship between such breach of duty and injury to the patient; and (4) the existence of damages that flow from the injury such that the legal system can provide redress.[57]

Although team physicians are frequently subjected to medical malpractice-based complaints from player-patients, there is very little

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53. *See id.* at 263 (explaining appeal procedures after receiving neutral physician report and arbitration procedures if grievance does not settle).

54. *See* Craig A. Isaacs, *Conflicts of Interest for Team Physicians: A Retrospective in Light of* Gathers v. Loyola Marymount University, 2 ALB. L.J. SCI. & TECH. 147, 152 (1992) (noting that liability for injuries "sustained during sports activities is the area generating the greatest amount of litigation").

55. *See* Michael Landis, *The Team Physician: An Analysis of the Causes of Action, Conflicts, Defenses, and Improvements*, 1 DEPAUL J. SPORTS L. & CONTEMP. PROBS. 139, 140 (2003) (explaining that medical malpractice, fraudulent misrepresentation or concealment of medical information, and intentional infliction of emotional distress are common lawsuits filed against team physicians).

56. *See* B. Sonny Bal, *An Introduction to Medical Malpractice in the United States*, 476 CLINICAL ORTHOPAEDICS & RELATED RSCH. 339, 340 (2008) (describing medical malpractice as "specific subset of tort law" that deals with negligence). Medical malpractice typically deals with negligence, which is defined as "conduct that falls short of a standard." *See id.* (explaining that negligence in medical malpractice actions looks to "reasonable" person standard, which is "standard of reasoned conduct that a person in similar circumstances would do, or not do, to protect another person from a foreseeable risk of harm").

57. *See id.* at 342 (listing four elements that must be met for plaintiff to have successful medical malpractice lawsuit).

case law that governs claims regarding professional sports teams and medical malpractice.[58]

a. Team Physicians' Duty to Player-Patients

The first element to prove in a medical malpractice case against a team physician is that the team physician had a duty to the patient-player.[59] Duty is defined as an "obligation to which the law will give recognition and effect to conform to a particular standard of conduct toward another."[60] In the context of team physicians, a physician-patient relationship is developed through either express or implied consent; both types of consent suffice because of team physicians' unique employment model.[61] For example, with team physicians, a physician-patient relationship may be formed through a general duty of physicians to act under a third-party beneficiary theory or if a physician begins to perform medical services for the patient's benefit.[62] Moreover, when a physician is employed by a

58. *See* Durao, *supra* note 31, at 37 (indicating that lack of specific case law distinguishing medical malpractice claims in professional sports means these elements must still be met in claims against professional sports team physicians).

59. For further discussion of elements to prove a prima facie case for medical malpractice in the United States, see *supra* notes 54–58 and accompanying text.

60. See Landis, *supra* note 55, at 140 (citing Joseph H. King, Jr., *The Duty and Standard of Care for Team Physicians*, 18 HOUS. L. REV., 657, 661 (1981)) (explaining that duty can be proven by showing there was physician-patient relationship between patient and defendant). Even if team physicians are not hired by the athlete themselves, duty may still exist when "the physician contracts with or is paid by a third party." *See id.* at 140–41 (citing Joseph H. King, Jr., *The Duty and Standard of Care for Team Physicians*, 18 HOUS. L. REV., 657, 664 (1981)) ("A physician who performs medical services for a team will owe a duty of care to the athletes on that team, but only within the scope of the medical services offered.").

61. *See id.* at 140 (explaining that physician-patient relationship requires consent, and duty can be imposed on team physicians through explicit or implied consent of player-patient).

62. *See* Twila Keim, *Physicians for Professional Sports Teams: Health Care Under the Pressure of Economic & Commercial Interests*, 9 SETON HALL J. SPORT L. 196, 201 (1999) (explaining that consensual physician-patient relationships can be developed in two ways). A third-party beneficiary theory imposes a duty on anyone who performs services for another individual's benefit. *See id.* (citing Scott Polsky, *Winning Medicine: Professional Sports Team Doctors' Conflicts of Interest*, 14 J. CONTEMP. HEALTH L. & POL'Y 503, 507 (1998)) ("A relationship may be established through a general duty, created pursuant to a third-party beneficiary theory, or through tort theory in which a duty is imposed upon anyone who begins to perform services for another's benefit."); *see also* Scott Polsky, *Winning Medicine: Professional Sports Team Doctors' Conflicts of Interest*, 14 J. CONTEMP. HEALTH L. & POL'Y 503, 507–08 (1998) (explaining third-party beneficiary theory occurs when physician's treatment occurs for purpose of "gaining information for third party"). Previously, courts have held that in these situations there is typically no duty of care owed to the patient, except if there are "dangerous conditions" or the exam was being conducted for therapeutic purposes. *See* Polsky, *supra* note 62, at 508 (explaining duty of care owed in third-party beneficiary situations). Courts have adopted a broader interpretation of duty, and in some cases, have found that physicians owe a duty of care even if the exam is being

team, they "owe a duty of care to the athletes on that team, but only within the scope of the medical services offered."[63]

In addition to mandating that team physicians provide proper medical treatment, the NFL CBA sets out other specific duties imposed on physicians acting in the role of a team physician.[64] A team physician also must provide the player-patient with all information regarding the player-patient's condition so that they can make an informed decision about their medical care and treatment.[65] In fact, team physicians have been held liable for violating the doctrine of informed consent in instances where physicians failed to fully and adequately disclose diagnostic information; however, the imposition of this duty could further decrease liability for team physicians in medical malpractice actions.[66]

After the physician's duty has been determined, there must be an inquiry into the standard of care that the team physician owes the player-patient.[67] Standard of care refers to "the level and type of care that a reasonably competent and skilled health care professional, with a similar background and in the same medical community, would have provided under the circumstances that led to the alleged malpractice."[68] Sports medicine is a highly specialized field, and

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conducted on behalf of a third-party. *See id.* (citing Betesh v. United States, 400 F. Supp. 238, 245–47 (D.D.C. 1974)) (explaining that courts have adopted broader interpretation of what duty is owed pursuant to third-party beneficiary theory).

63. *See* Landis, *supra* note 55, at 141 (describing limited nature of duty owed by team physicians to athletes on team they work for).

64. *See, e.g.,* NFL & NFLPA CBA, *supra* note 43, at 213 (designating duties to team physicians such as clearing players to return to play, diagnosing and treating players' conditions, disclosing information regarding player-patients' physical conditions).

65. *See* Landis, *supra* note 55, at 142 (acknowledging that team physicians are bound by doctrine of informed consent so player-patients may make informed decisions about their health).

66. *See* Krueger v. San Francisco Forty-Niners, 234 Cal.Rptr. 579, 584 (Ct. App. 1987) (holding appellant football player established negligence and fraudulent concealment of team physicians because they failed to adequately inform player of risk in continuing to play with damaged left knee).

67. *See* Landis, *supra* note 55, at 141 (implying standard of care by team physicians has been area of debate because of diverse specialties encapsulated within field of sports medicine). The complex character of this field has led scholars to develop different ideas about what the standard of care for these team physicians should be. *See id.* (discussing both "optimal" and "minimal" standards of care and that determination of team physicians' liability should be made on case-by-case basis).

68. *See* David Goguen, *What Is the "Medical Standard of Care" in a Medical Malpractice Case?*, NOLO, https://www.nolo.com/legal-encyclopedia/what-the-medical-standard-care-malpractice-case.html [https://perma.cc/7J67-KBLU] (last visited Jan. 26, 2024) (providing definition of medical standard of care). Typically, the standard of care is akin to a reasonable person standard in which one looks at whether a "similarly skilled health care professional" would have provided the "same treatment under the same, or similar, circumstances." *See id.* (discussing questions to ask when determining appropriate standard of care).

because of the unique aspects of litigation surrounding it, courts have not yet adopted a "one-size-fits-all" standard of care for team physicians; in fact, many courts are split on what existing standard, if any, should be applied to these cases.[69] Currently, the standard of care expected of a team physician is a highly fact-dependent inquiry.[70] As a result, courts usually look to whether the team physician's conduct reasonably occurs in their medical specialty.[71]

b. Breach

The second element to prove in a medical malpractice case against a team physician is that the team physician breached their duty to the patient-player.[72] Breach occurs when "the physician's conduct, by way of an act or omission, [falls] below the standard of care."[73] Expert testimony is required to determine whether there was a breach of the standard of care.[74] As previously discussed, there must be a physician-patient relationship established before a breach may be found.[75]

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69. *See* Keim, *supra* note 62, at 202 (describing debate among medical professionals regarding appropriate standard of care for team physicians). Some medical professionals argue that team physicians "should be held to a 'reasonably competent general practitioner' standard unless he or she provides services performed exclusively by specialists or holds himself/herself out as possessing special competence in sports medicine." *See id.* (describing one side of debate regarding appropriate standard of care). Additionally, some other medical professionals argue that "team physicians should be treated as specialists observing 'fundamentals that all practicing specialists in sports medicine should know, based on the types of athletes the physician treats." *See id.* (explaining alternate view of appropriate standard of care); *see also* Tara R. Di Luca, *A Whole Different Ballgame . . . or Is It?: Medical Malpractice and the Modern-Day Athlete*, NYSBA J. 35 (2008), https://schiavettilaw.com/wp-content/uploads/2022/02/NYSBA-article.pdf [https://perma.cc/L6HD-QY46] (explaining view that "[c]ourts have been hesitant to apply the traditional locality rule in favor of a more uniform national standard, reasoning that appropriate treatment should not vary with the geographic location of where the treatment is rendered" since sports teams travel to different areas of United States).

70. *See* Landis, *supra* note 55, at 141 (indicating that case-by-case analysis depends on treatment and medical care provided to specific player-patient before court settles on appropriate standard of care).

71. *See id.* (acknowledging that courts do not look at sports medicine as specialty, but instead look to "good medical practice" standard, adherence to which is measured by comparing conduct of physician in question to common practice of other physicians in same specialty).

72. For further discussion of elements to prove a prima facie case for medical malpractice in the United States, see *supra* notes 54–58 and accompanying text.

73. *See id.* at 143 (emphasizing that one must determine that there was duty owed before determining whether standard of care was breached).

74. *See id.* (explaining role of expert testimony in proving breach of standard of care in medical malpractice cases).

75. *See* Di Luca, *supra* note 69, at 35 (explaining precedent that establishes physician-patient relationships). In *Scottland v. Duva Boxing LLC*, a physician-patient relationship was established because the defendants were "ringside physicians" during a match and had a "duty to exercise reasonable medical care

Many states require expert testimony regarding the professional standard of care to be provided by a licensed health care provider that is a specialist in a same or similar specialty as the defendant-physician.[76] Besides having restrictions on *who* may testify as an expert witness, the United States Supreme Court has imposed a restriction on *what* an expert witness may testify about in a trial.[77] In 1993, the seminal decision in *Daubert v. Merrell Dow Pharmaceuticals Inc.*,[78] explicitly permitted trial judges to act as gatekeepers of scientific evidence, by requiring judges to follow a list of factors to assess the "methodology and reasoning behind an expert's opinions."[79] This decision effectively overruled the 1923 decision in

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and to provide an ongoing medical diagnosis of the athlete's physical condition throughout the match." *See id.* (citing Scottland v. Duva Boxing LLC, 2005 N.Y. Misc. LEXIS 3513 (2005)). Moreover, in *Kleinknecht v. Gettysburg College*, the Court found the college had a physician-patient relationship with the athletes since the college recruited them as players. *See id.* at 36 (citing Kleinknecht v. Gettysburg Coll., 989 F.2d 1360 (3d Cir. 1993)) (holding that mere recruitment created duty to provide "prompt emergency medical services").

76. *See, e.g.*, Weiss v. Pratt, 53 So.3d 395, 399 (Fla. Dist. Ct. App. 2011) (explaining that Florida's expert testimony statute requires specialization in same specialty or similar specialty and "devoted professional time during last [three] years" to clinical practice, instruction of students, or research program at accredited health school). *Weiss* further indicated that an emergency room physician was permitted to opine an expert report regarding a volunteer team physician's treatment as the emergency room physician had the necessary expertise. *See id.* (holding trial court did not err in allowing emergency room physician to render expert opinion); *see also Medical Testimony*, AM. MED. ASS'N, https://code-medical-ethics.ama-assn.org/ethics-opinions/medical-testimony [https://perma.cc/8AJM-UZU7] (last visited June 29, 2024) (explaining that expert witness ethical code requires expert to testify in areas where they received "appropriate training and recent, substantive experience and knowledge").

77. *See* Daubert v. Merrell Dow Pharm. Inc., 509 U.S. 579, 597 (1993) (holding that "general acceptance test" used by *Frye* is not precondition to "admissibility of scientific evidence under Federal Rules of Evidence"). *Daubert* replaced the "general acceptance" test that was the rule originally announced in *Frye*. *See id.* at 585–90 ("*Frye* made 'general acceptance' the exclusive test for admitting expert scientific testimony. That austere standard, absent from, and incompatible with, the Federal Rules of Evidence, should not be applied in federal trials."). The general acceptance test stated in *Frye* required that the scientific community "generally accepts" the scientific principle or methodology an expert witness relies upon for the expert testimony may be admitted as evidence. *See id.* at 586 (citing Frye v. United States, 293 F. 1013, 1014 (D.C. Cir. 1923)) ("Just when a scientific principle or discovery crosses the line between the experimental and demonstrable stages is difficult to define. Somewhere in this twilight zone the evidential force of the principle must be recognized, and while courts will go a long way in admitting expert testimony deduced from a well-recognized scientific principle or discovery, the thing from which the deduction is made must be sufficiently established to have gained general acceptance in the particular field in which it belongs.").

78. Daubert v. Merrell Dow Pharm. Inc., 509 U.S. 579 (1993).

79. *See id.* at 593–94 (summarizing test to determine admissibility of scientific evidence). In *Daubert*, the Supreme Court provided the following factors to determine whether an expert's methodology is valid:

> (1) whether the technique or theory in question can be and has been tested; (2) whether it has been subjected to publication and peer review;

Frye v. United States[80] in which the court held that expert testimony is admissible so long as the methods the opinion is based upon are generally accepted as reliable in the scientific community.[81] However, some state courts still follow the standard established by *Frye,* for limiting evidence and expert testimony.[82]

c. Causation and Harm

The third element to prove in a medical malpractice case against a team physician is that the team physician's breach of duty was the actual cause or proximate cause of the harm to the patient-player.[83] Causation is defined as a "reasonable connection between the negligent act committed by the physician and the damages that the athlete suffered."[84] Causation is determined by either cause in fact or proximate cause.[85] Finally, the last element to prove in a medical malpractice case against a team physician is that the patient-player was injured because of the negligence.[86] Harm, the final element of a medical malpractice lawsuit, refers to the damage sustained by

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> (3) its known or potential error rate; (4) the existence and maintenance of standards controlling its operation; and (5) whether it has attracted widespread acceptance within a relevant scientific community.

See id. (discussing test used to determine admissibility of expert testimony); *see also* Daubert *Standard*, CORNELL UNIV. L. SCH.: LEGAL INFO. INST. (Aug. 2023), https://www.law.cornell.edu/wex/daubert_standard [https://perma.cc/LL7N-GJFC] (affirming that "*Daubert* Standard supplanted the *Frye* Standard" in federal court).

80. Frye v. United States, 293 F. 1013 (D.C. Cir. 1923).

81. *See id.* at 1014 (holding that expert testimony must be based on "well-recognized scientific principle or discovery" and should have general acceptance in scientific community); *see also* Anjelica Cappellino, Daubert *vs.* Frye*: Navigating the Standards of Admissibility for Expert Testimony*, EXPERT INST. (Apr. 11, 2022), https://www.expertinstitute.com/resources/insights/daubert-vs-frye-navigating-the-standards-of-admissibility-for-expert-testimony/ [https://perma.cc/99E7-XS3R] (explaining that federal courts exclusively follow standard as set forth in *Daubert* because *Frye* was inconsistent with Federal Rules of Evidence).

82. *See Frye*, 293 F. at 1014 (holding that any methodology employed by expert witness that does not have significant scientific recognition is inadmissible). Although the *Frye* standard has been superseded by the *Daubert* standard federally and has been "abandoned by many states," there are still states that follow *Frye*. *See* Frye *Standard*, CORNELL UNIV. L. SCH.: LEGAL INFO. INST. (Dec. 2022), https://www.law.cornell.edu/wex/frye_standard [https://perma.cc/UES5-RVZQ] (explaining *Frye* standard criticized for "preventing reliable technology from being used in courts just because majority of field has yet to adopt the technology").

83. For further discussion of elements to prove a prima facie case for medical malpractice in the United States, see *supra* notes 54–58 and accompanying text.

84. *See* Di Luca, *supra* note 69, at 37 (explaining that physician's negligence must have caused athlete's harm for there to be causation).

85. *See* Landis, *supra* note 55, at 145 (describing two ways causation may be found).

86. For further discussion of elements to prove a prima facie case for medical malpractice in the United States, see *supra* notes 54–58 and accompanying text.

an athlete.[87] Expert testimony is required to establish damages and ensure there is a proper award based on the harm.[88]

B. The Effects of Medical Malpractice Litigation in Other Sports Leagues

While the NFL has seen some of the highest medical malpractice judgments against its team physicians in recent years, other sports leagues have not escaped the recent boom of team physician-based medical malpractice suits.[89] However, the degree of impact felt by the other sports leagues due to recent booms in medical malpractice litigation varies because of different liability and conflict-limiting provisions in their CBAs.[90] Although the NFL has some of the best policies regarding player health, there are still ways to improve these policies to alleviate the substantial financial strain from medical malpractice litigation felt by its team physicians.[91]

1. *National Basketball Association (NBA)*

The NBA has thirty member teams that participate in the professional basketball league.[92] Like the NFL, the players of the NBA have formed a players' association, which is "a labor union empowered with certain rights and responsibilities under federal labor laws."[93] Compared to the NFL, the NBA has significantly lower injury reports per game.[94]

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87. *See id.* at 145 (explaining that athlete will receive damages to compensate for harm suffered because of team physicians' negligence).

88. *See id.* at 145 (emphasizing importance of expert testimony to prove amount of damages requested due to harm suffered by plaintiff).

89. For further discussion of NFL medical malpractice verdicts, see *supra* notes 1–10 and accompanying text.

90. For further discussion of similarities to other sports leagues' CBAs, see *infra* notes 92–126 and accompanying text.

91. *See* Christopher R. Deubert, I. Glenn Cohen & Holly Fernandez Lynch, *Comparing Health-Related Policies & Practices in Sports: The NFL and Other Professional Leagues*, PETRIE-FLOM CTR. FOR HEALTH L. POL'Y, BIOTECHNOLOGY & BIOETHICS 13 (2017), https://footballplayershealth.harvard.edu/wp-content/uploads/2017/05/Harvard-Comparative-League-Analysis-5.15.17.pdf [https://perma.cc/PNE2-VHGW] (arguing reports "generally conclude" that "NFL's policies concerning player health appear superior" to other professional sports leagues, however, there are areas for improvement).

92. *See id.* at 14 (indicating that NBA is both "structurally and legally" similar to NFL).

93. *See id.* (explaining that players' unions are interested in protecting and advancing players' individual rights, including their right to proper medical care).

94. *See id.* at 17 (discussing that mean number of injuries suffered in NBA is 0.16 per game, compared to NFL's 5.90 injuries per game).

The NBA's CBA requires that each team physician is board certified in an area of medical expertise, has completed a fellowship in sports medicine or similar qualifications, and has at least five years of clinical experience.[95] All NBA team physicians are hired individually by a specific team, and the CBA explicitly grants all teams discretion regarding selection of "any doctor, hospitals, clinics, health consultants, or other health care providers."[96] Like the NFL's CBA, the NBA's CBA permits players to obtain a second medical opinion by a physician of their choosing.[97]

Although the NBA's CBA does not permit team physicians to receive a sponsorship for their services, there are still concerns regarding potential conflicts of interest.[98] For example, a team

95. *See* NBA & NBA PLAYERS' ASS'N, COLLECTIVE BARGAINING AGREEMENT 380 (July 2023) [hereinafter NBA & NBPA CBA], https://ak-static.cms.nba.com/wp-content/uploads/sites/4/2023/06/2023-NBA-Collective-Bargaining-Agreement.pdf [https://perma.cc/3GRD-32XR] (explaining educational and experiential requirements for NBA team physicians). The NBA's CBA also requires other certifications, such as training in "Basic Life Support, Basic Trauma Life Support, Advanced Cardiac Life Support, or Advanced Trauma Life Support." *See id.* (providing other training requirements to be NBA team physician). The NBA's CBA also reserves the right to implement requirements regarding game coverage. *See id.* (explaining NBA's CBA may require attendance at every home game of team physician who is board certified in orthopedic surgery and team physician who is board certified in "internal medicine, family medicine, or emergency medicine").

96. *See id.* (explaining hiring requirements and parameters for NBA team physicians). Interestingly, the NBA's CBA explicitly prohibits teams from having sponsorship relationships with any health care provider. *See id.* (indicating any type of sponsorship relationship with team is prohibited to align with NBA CBA's goal of "providing high quality care to all of its players").

97. *See id.* at 386 (explaining that before obtaining second opinions regarding medical diagnoses, NBA players must explain initial course of treatment for medical issue). The CBA specifically states:

> [P]revented the player from participating in a Regular Season, Play-In, or playoff game for two (2) weeks or more; (ii) in the opinion of a Team physician for the player's Team, is more likely than not to prevent the player from being able to participate in an NBA game for two (2) weeks or more (or during the off-season, from participating in competitive basketball without restriction for two weeks or more); (iii) in the opinion of the Team physician will not be significantly aggravated by the player continuing to participate in NBA games (or during the offseason participating in basketball without restriction) when the player reasonably believes that continued participation will significantly aggravate his injury, illness, or other medical condition; (iv) results in direction from the Team physician that the player should undergo surgery; or (v) results in direction from the Team physician that the player should not undergo surgery when the player reasonably believes that surgery is necessary for the injury, illness, or other medical condition.

See id. (quoting NBA CBA language regarding requirements for obtaining second opinion on medical diagnosis). *But see* NFL & NFLPA CBA, *supra* note 43, at 219 (indicating that NFL has no other requirements for player to obtain second opinion regarding injury diagnosis).

98. For further discussion of the potential for conflict of interest issues for team physicians based on the current arrangement of the role, see *infra* notes 127–144 and accompanying text.

physician is permitted to disclose "relevant" medical information about a player to the following individuals or groups:

> (i) the General Manager, coaches, and trainers of the Team by which such player is employed, (ii) any entity from which any such Team seeks to procure, or has procured, an insurance policy covering such player's life or any disability, injury, illness, or other medical condition such player may suffer or sustain, and (iii) . . . the media or public on behalf of the Team.[99]

By permitting open disclosure of these medical records to team personnel, the NBA's CBA implicitly causes distrust between athletes and physicians, thus affecting the quality of the patient-physician relationship.[100] This provision, compounded with the provision requiring numerous factors be met before an athlete can obtain a second opinion, further harms the relationship between the physician and the athlete because it creates a false expectation that the athlete must blindly follow all decisions made by the team physician.[101]

2. *Major League Baseball (MLB)*

The MLB has thirty member clubs that participate in its professional baseball organization.[102] Like the NFL, the players of the MLB have formed a players' association.[103] Compared to other sports leagues' CBAs, the MLB's CBA does not expressly state the qualifications required to serve as a team physician, nor clarify the duties related to employment for their team physicians.[104] Notably, while the MLB's CBA makes references to its club physicians, the only

99. *See* NBA & NBPA CBA, *supra* note 95, at 380–407 (explaining that team personnel may receive medical information about NBA players).

100. For further discussion of creating change to preserve the relationship between the team physicians and NFL players, see *infra* notes 141–170 and accompanying text.

101. For further discussion of the elements that create a distrusting relationship between the team physicians and NFL players, see *infra* notes 127–144 and accompanying text.

102. *See* Deubert et al., *supra* note 91, at 14 (indicating MLB has been operating for more than century and is one of "Big Four" sports leagues); *see also MLB Teams*, ESPN, https://www.espn.com/mlb/teams [https://perma.cc/AHH7-RWRS] (last visited Sept. 13, 2024) (listing thirty MLB teams and their locations).

103. *See id.* (illustrating players' unions are involved in protecting and advancing players' rights, which include medical care, and health and safety while playing).

104. *See generally* MLB & MLB PLAYERS' ASS'N, COLLECTIVE BARGAINING AGREEMENT 55–66 (Mar. 10, 2022) [hereinafter MLB & MLBPA CBA], https://www.mlbplayers.com/_files/ugd/4d23dc_d6dfc2344d2042de973e37de62484da5.pdf [https://perma.cc/932Q-FLMR] (acknowledging protections in place for Major League Baseball Players regarding health and safety, but no indications of team physician's role in MLB's CBA).

medical professional that it explicitly references training requirements or game attendance requirements for is athletic trainers.[105]

Like the NFL's CBA, the MLB's CBA also permits players to obtain a second opinion on any "employment-related illness or injury being treated by the Club Physician."[106] However, the MLB's CBA does not permit its players to choose the physician for a second opinion.[107] Instead, they must choose from a pre-set list of medical providers.[108] While there is an option for MLB players to choose a physician outside of the selected list of physicians, the player will be financially responsible for these costs unless both the player and the team devise a written agreement establishing that the MLB will cover the costs.[109]

3. *National Hockey League (NHL)*

Like the NFL, the players of the NHL have formed a players' association, known as the National Hockey League Players Association (NHLPA).[110] Compared to other sports leagues, including the NFL, the NHL's CBA imposes extensive restrictions on team

105. *See id.* at 58–59 (explaining requirements and duties for medical professional to be certified as athletic trainer in MLB's CBA). The MLB's CBA requires that each team employ two certified athletic trainers who are required to travel with the team to games as needed. *See id.* (acknowledging that individual must be certified by National Athletic Trainers Association, Canadian Athletic Therapists Association, or be physical therapist certified by "appropriate state authority" to be considered "certified athletic trainer").

106. *See id.* at 57 (explaining second medical opinion policy and requiring MLB to provide list of accepted medical specialists by specialty for second medical opinions). Prior to receiving a second medical opinion, an injured player must inform "the Club in writing" of their intention to receive this second opinion and provide the name of the physician "who will be performing the diagnosis and medical evaluation." *See id.* (indicating that MLB will cover costs of evaluation for second medical opinion).

107. *See id.* (explaining that MLB compiles list of physicians that MLB players may go to seek second opinion). The MLB has a list of "at least two board-certified physicians . . . [in] each specialty in each of the geographic regions." *See id.* (acknowledging Medical Advisory Committee, along with selected medical professional chosen by MLB, will review and update list of specialists annually).

108. *See id.* (stating that Commissioner's Medical Advisory Committee, along with medical professionals selected by MLB, compile list of specialists that may perform second opinion for athletes).

109. *See id.* (explaining that injured player must provide, in writing, notice regarding player's intention to seek care from medical specialist who is not approved by league). The MLB will only permit this second medical opinion by an unauthorized physician if there is a prior agreement between the player and the team. *See id.* (summarizing differences between obtaining second medical opinion from approved physician and obtaining second opinion from unapproved physician).

110. *See* Deubert, et al., *supra* note 91, at 14 (explaining NHL also has players' union known as NHLPA).

physicians.[111] In fact, the CBA begins with a "quality of care" excerpt which states that each club team must provide its players with "high quality health care."[112]

Additionally, the quality of care excerpt includes a paragraph regarding the team physicians' allegiance expectations, emphasizing that the team physician's duty is to the patient, despite being hired by the club to treat players.[113] The CBA also requires that any disputes regarding the quality of care received must be referred to the NHL/NHLPA Joint Health and Safety Committee for "discussion and potential resolution."[114] However, based on the language of the quality of care excerpt, it is unclear whether medical malpractice claims must be reviewed by this Committee prior to an injured player initiating a lawsuit against a team physician.[115]

Like other sports leagues, including the NFL, the NHL's CBA requires each club to have at least "two team physicians in attendance at all home games."[116] Each team physician must also be board certified and have completed a fellowship in sports medicine.[117]

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111. *See* NHL & NHL PLAYERS' ASS'N, COLLECTIVE BARGAINING AGREEMENT 213–22 (Sept. 16, 2012) [hereinafter NHL & NHLPA CBA], https://cdn.nhlpa.com/img/assets/file/NHL_NHLPA_2013_CBA.pdf [https://perma.cc/8Q9S-SRFY] (summarizing NHL CBA's expectations regarding "quality of care" for team physicians). The CBA sets forth that each team will provide its players with "high quality health care," implying that team physicians are held to a higher standard of care based on their occupation as an NHL team physician. *See id.* at 176 (indicating "primary professional duty" of all medical staff is to player-patient, regardless of whether team physician is employed by team); *see also Collective Bargaining Agreement*, NHL PLAYERS' ASS'N, https://www.nhlpa.com/the-pa/cba [https://perma.cc/MR9K-GQKM] (last visited Mar. 17, 2024) (acknowledging that although CBA was ratified in 2012, both NHLPA and NHL negotiated four-year extension to 2012 CBA, and therefore, CBA is extended through 2025-2026 season).

112. *See* NHL & NHLPA CBA, *supra* note 111, at 176 (summarizing quality of care expected by team physicians and other health professionals to NHL players). Interestingly, the quality-of-care statement explicitly states that team physicians are required to "perform complete and objective examinations and evaluations" on behalf of the club; however, the team physician is still "subject to all professional and legal obligations vis-à-vis the player-patient." *See id.* (providing key details regarding standard of care for NHL team physicians and that they still owe player-patient higher duty despite being employed by NHL team and completing certain duties).

113. *See id.* (acknowledging unique employment relationship between team physicians and NHL team).

114. *See id.* (explaining that any disputes regarding quality of care are required to go through discussion with NHL/NHLPA Joint Health and Safety Committee).

115. *See id.* (implying that procedures within NHL/NHLPA Joint Health and Safety Committee must be followed before state tort claim is filed).

116. *See id.* at 176–77 (explaining minimum attendance requirements for team physicians regarding games).

117. *See id.* (summarizing educational requirements for team physicians). The NHL's CBA also requires that at least one team physician "have successfully completed hockey-specific trauma management training or Advanced Trauma Life Support training during the previous three (3) years." *See id.* at 176 (stating additional training requirements aside from being certified in certain specialty).

Although there are many similarities between the NFL's CBA and NHL's CBA regarding requirements for their team physicians, unlike the NFL's CBA, the NHL's CBA creates stricter attendance requirements for team physicians during its games.[118] For example, all team physicians must be "seated in close proximity to . . . the Players' bench with immediate access to the bench and ice surface . . ."[119] Additionally, during game play, at least one team physician is required to observe the game on a "reliable live television feed, provided rapid access to the bench and ice surface is not compromised."[120]

The NHL's CBA also requires that players complete a medical authorization, which permits the release of a player's medical information to club personnel for purposes relating to the player's employment.[121] This provision directly promotes a sense of distrust between the team physicians and the patient-athlete.[122] Notably, this provision forcing players to consent to the release of their medical information is not in the NFL's CBA.[123]

Finally, like the NFL's CBA, the NHL's CBA allows players to obtain a second medical opinion "regarding a diagnosis made by a team physician or a course of treatment . . . prescribed by a team physician."[124] However, unlike the NFL's CBA, a player is required to

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118. *See id.* (explaining requirements for team physicians during game play).

119. *See id.* (acknowledging specific locations NHL team physicians must be present during game to ensure responsibilities are met).

120. *See id.* (implying having team physician watch game play ensures "rapid access" to players' bench and ice surface, therefore, ensuring quick response to any injury during game).

121. *See id.* at 178 (explaining policy for medical authorizations to NHL players, including disclosure of medical information to NHLPA and NHL team "as reasonably required for professional sports operations and related to the Player's employment as an NHL hockey player"). Medical disclosure is permitted if it relates to player's employment but is not permitted if it does not relate to employment or if player did not provide "express, prior, written consent." *See id.* (explaining procedures for medical disclosure, and when medical information may be disclosed if dispute arises regarding medical care).

122. For further discussion of distrusting relationship between team physicians and athletes, see *infra* notes 127–170 and accompanying text.

123. *See generally* NFL & NFLPA CBA, *supra* note 43, at 213 (acknowledging that no provision for medical disclosure is present in various contexts, thus, permitting team physicians to disclose player's medical information based on "federal, state, and local requirements, including all ethical rules and standards established by any applicable government"). It is important to note that NFL allows medical information to be disclosed to a "coach or other Club representative, whether or not such information affects the player's performance or health." *See id.* (implying authority to disclose medical information by team physicians is broad).

124. *See* NHL & NHLPA CBA, *supra* note 111, at 181–82 (explaining procedure for second medical opinions). Like the MLB, the NHL provides a list of "medical specialists" that have been "mutually agreed" upon by the Joint Health and Safety Committee. *See id.* at 181 (acknowledging that NHLPA is involved with selection of physicians for second medical opinion). Moreover, if there is "no consensus on diagnosis and/or course of treatment," then the injured player may elect to obtain

seek a second opinion from a list of medical specialists provided by the Joint Health & Safety Committee.[125] Although the NHL's CBA intends to allow the players to have a second opinion from another physician, the NHL's CBA is restraining free choice by prohibiting its athletes in allowing for a second opinion from a physician of their own choosing.[126]

III. TACKLING THE DEFENSE: PROBLEMS WITH HEIGHTENED LIABILITY OF TEAM PHYSICIANS AND SOLUTIONS TO MINIMIZE NEGATIVE EFFECTS OF MEDICAL MALPRACTICE LITIGATION

A. Distrust is Our Only Defense Against Betrayal: Problems with the Current Team Physician Model and Potential Solutions to Alleviate Conflict of Interest Issues

Despite the clear purpose for team physicians in the NFL, the dueling loyalties plaguing their roles could negatively impact the quality of care provided to player-patients.[127] Some of these potential conflicts have been questioned publicly by NFL players.[128] As a result, these issues, such as conflicting responsibilities to both the athlete and the team and concerns regarding potential notoriety relating to their services, can create a sense of distrust between the athletes and the team physicians.[129]

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a third medical opinion. *See id.* at 182 (explaining process for NHL player to obtain third medical opinion). A third opinion shall be sent to the NHL player, the physician who provided the second medical opinion, and the regular team physician. *See id.* (noting process for third opinion).

125. *See id.* at 181 (stating second medical opinion physicians are listed by "specialty and by geographic region").

126. *See* NFL & NFLPA CBA, *supra* note 43, at 219 (acknowledging that besides being able to choose their second medical opinion physician, NFL players have right to follow reasonable medical advice provided by second opinion physicians regarding "diagnosis of injury, surgical and treatment decisions, and rehabilitation and treatment protocol"). Although the NFL player may choose to follow medical advice from the second physician, they still must first consult the team physician. *See id.* (implying team physician still involved in medical advice process).

127. *See* Mitten, *supra* note 27, at 131–32 (arguing that team physicians are subjected to pressure from player-patients and team to clear player-patient to return to play).

128. *See Former Eagles Captain Chris Maragos Awarded $43.5 Million in Lawsuit Over Career-Ending Injury*, THE ATHLETIC (Feb. 13, 2023), https://theathletic.com/4192466/2023/02/13/chris-maragos-lawsuit-knee-injury/ [https://perma.cc/M76G-CZUM] (stating Maragos' verdict "serves as a reminder that any team's doctor, in any sport, who jeopardizes the well-being of its players due to contractual obligations or financial incentives, will be held accountable for their misconduct").

129. *See* Mitten, *supra* note 27, at 140 (arguing that team physicians experience pressures due to conflicting obligations to both athletes and team); *see also* Christopher R. Deubert, I. Glenn Cohen & Holly Fernandez Lynch, *Protecting and Promoting the Health of NFL Players: Legal and Ethical Analysis and Recommendations,*

The current team physician employment model has created the appearance of "dual loyalty" between the physician and the team, and the physician and the player.[130] Although the NFL's CBA minimizes the effects of "dual loyalty" by allowing NFL players to have various options in their care, the hiring process for team physicians is still exclusively conducted by the teams, not the players.[131] The biggest decision that a team physician is involved in pertains to whether an athlete should be permitted to return to activity; however, a team's goal is to have all team members playing.[132] As a result, even though the team physician is required to provide medical advice to protect the athlete's health, they still face pressure from their employer to allow the athlete to return to play.[133] In fact, a team physician might even face further pressure from the players themselves to clear them to return to activity as quickly as possible, which presents additional challenges to the physician-patient relationship.[134]

Because of these competing interests and a team physicians' allegiance to the team, it is likely that team physicians sometimes make decisions that are not in the player's best interests.[135] This is

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HARV. U. L. SCH. 11, 16 (2016) (arguing there are "inherent structural conflict[s] of interest" among team physicians and players, which create legal and ethical issues that could impact player health). Although the presence of this conflict of interest is "not a moral judgment about [team physicians] as competent professionals or devoted individuals, but rather a simple fact of the current organizational structure of their position in which they simultaneously perform at least two roles that are not compatible." *See* Deubert et al., *supra* note 129, at 16 (acknowledging that current "ethic codes and legal requirements" do not eliminate conflict or guarantee players receive best possible healthcare).

130. *See* Steve P. Calandrillo, *Sports Medicine Conflicts: Team Physicians vs. Athlete-Patients*, 50 ST. LOUIS U. L.J. 185, 190 (2005) (implying employment relationship between team physician and hiring team creates "allegiance" to hiring team, which conflicts with duty of loyalty owed to patient-players).

131. For further discussion of the CBA protections for NFL players and their medical care, see *infra* notes 231–244 and accompanying text.

132. *See* Mitten, *supra* note 27, at 140 (explaining role of team physician in determining whether player should return to play after injury).

133. *See 1.2.5 Sports Medicine*, AMA CODE OF MED. ETHICS 1–3 (1983), https://code-medical-ethics.ama-assn.org/sites/amacoedb/files/2022-08/1.2.5%20Sports%20medicine%20–%20background%20reports.pdf [https://perma.cc/K4QA-EK98] (acknowledging that team physicians should "not allow desires of spectators, promoters of event, or even injured individual to govern decision about whether to remove participant from event").

134. *See id.* (explaining no other interest should impact team physician's decision to clear player to return to play).

135. *See* Calandrillo, *supra* note 130, at 190 (indicating that it is common for players to push for proactive clearance to return to play despite lingering safety concerns); *see also* Brad Mielke & Ivan Pereira, *NFL Under Fire Over Its Concussion Protocol Following Tua Tagovailoa's Back-to-Back Game Hits*, ABC NEWS (Oct. 3, 2022, 6:54 PM), https://abcnews.go.com/Sports/nfl-fire-concussion-protocol-tua-tagovailoas-back-back/story?id=90918691 [https://perma.cc/7QLK-U7SP] (explaining that Miami Dolphins' team physician was fired because of reckless decision to allow quarterback, Tua Tagovailoa, to return to play after concussion). The decision to

problematic in cases where the injured athlete does not appear to understand the magnitude of the decision to return to play.[136] For example, some players, especially in the NFL, who suffer a concussion might not understand the significance of their injury and the importance of taking time off to recover.[137]

The current team physician employment model has led to a sense of distrust between some NFL athletes and their doctors.[138] In some cases, athletes' distrust in their medical professionals is automatic because they are hired by their respective teams.[139] This attitude towards team physicians has also been adopted by the general public, especially in cases involving the NFL's concussion protocol.[140] The distrust between players and team physicians is further exacerbated when workers' compensation claims are involved; there have been many instances where teams have purposely attempted to

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let Tagovailoa to return to gametime so quickly was considered reckless by the public because he showed possible concussion symptoms, such as a lack of balance and stability, before being hospitalized after hitting his head on the turf field during a game against the Cincinnati Bengals. *See* Marcel Louis-Jacques, *Miami Dolphins QB Tua Tagovailoa Back in Concussion Protocol*, ESPN (Dec. 26, 2022, 5:17 PM), https://www.espn.com/nfl/story/_/id/35324355/miami-dolphins-qb-tua-tagovailoa-concussion-protocol-again [https://perma.cc/G8BW-JLAP] (explaining that Tagovailoa showed symptoms of concussion, but still stayed in game after being evaluated under NFL's concussion protocol and despite showing apparent concussion symptoms).

136. *See* Andrew D. Hohenstein, *Team Physicians: Adhering to the Hippocratic Oath or Just Plain Hypocrites?*, 19 MARQ. SPORTS L. REV. 579, 605 (2009) (explaining that players with severe injuries might not completely comprehend seriousness of circumstances). As a result, players might agree to treatment or other considerations that are not in their best long-term interest. *See id.* (noting effects of returning to play too quickly after severe injury, including incoherence and other exacerbated symptoms).

137. *See id.* at 593 (explaining that players with concussions typically do not understand consequences of returning to play too quickly, and risk aggravating their condition and causing further injury).

138. *See* Deubert et al., *supra* note 91, at 81–82 (summarizing interviews with former and current NFL players that illustrate lack of trust between team physicians and players regarding quality of care received). The players that were interviewed also expressed that the communications between "club medical staff and the coaches and general manger" pressure players to "withhold information" from team physicians. *See id.* ("Players often do not want to tell the medical staff that they are not healthy enough to practice for fear that the medical staff will relay that message to the general manager, with the suggestion that the general manager consider signing a potential replacement player.").

139. *See* Ike Swetlitz, *NFL Doctors' Conflicts of Interest Could Endanger Players, Report Says*, STAT NEWS (Nov. 17, 2016), https://www.statnews.com/2016/11/17/nfl-doctors-conflict-interest/ [https://perma.cc/G7QX-S2SZ] (indicating that athletes' distrust of team physicians stems from employment model).

140. *See* Ken Belson, Jenny Vrentas & Emmanuel Morgan, *Sideline Doctors Face Scrutiny After Hits to a Star Quarterback*, N.Y. TIMES (Oct. 8, 2022), https://www.nytimes.com/2022/10/07/sports/football/football-concussions-tagovailoa.html [https://perma.cc/U74C-UAAR] (summarizing general public's negative impression of medical care after Miami Dolphins' quarterback, Tua Tagovailoa's 2022 injury).

designate team physicians as "employees" when professional athletes are covered by state workers' compensation laws.[141] Additionally, since many team physicians are not highly compensated for their roles, they may use their affiliation with the NFL team as a marketing device to grow their independent medical practices.[142] In practice, the status granted to team physicians upon partnering with an NFL team permits them to market their private practices as affiliates of the NFL team, thus generating business by associating their independent practice with the professional team itself.[143] By doing this, teams undermine the relationship between the athlete and the team physician and effectively illustrate that the team physician is loyal to the team, thus resulting in conflicting allegiances between the team and professional athletes they treat.[144]

These conflicts between the team physician, the athletes, and the team may be alleviated in several ways.[145] First, the NFL must recognize that the relationship between team physicians and athletes is a collaborative relationship.[146] This relationship can be encouraged by promoting open communication between team physicians and the athletes, and making sure the player-patients are the team physicians' top priority.[147] By recognizing this collaborative relationship, the NFL will be able to encourage an open, trusting dynamic between the player and the team physician.[148] This open relationship

141. *See* Mitten, *supra* note 19, at 213 (explaining if team physician is designated as "employee," then NFL players cannot pursue state tort claims against them and must apply for workers' compensation, which presents further difficulties).

142. *See* Calandrillo, *supra* note 130, at 190 (explaining that team physicians are typically unpaid volunteers and are permitted to use their position with hiring team as marketing tool to develop and grow their private practices).

143. *See id.* at 195 (indicating that many team physicians market their roles as "endorsement" by NFL team).

144. *See id.* at 212 (arguing current relationship of team physician encourages feelings that team physician is "on the side" of team and NFL).

145. *See* I. Glenn Cohen, Holly Fernandez Lynch & Christopher R. Deubert, *A Proposal to Address NFL Club Doctors' Conflicts of Interest and to Promote Player Trust*, HASTINGS CTR. REP. (Nov. 21, 2016), https://www.ncbi.nlm.nih.gov/pmc/articles/PMC5215412/ [https://perma.cc/QTQ9-BT5X] (arguing that conflicts of interest with current team physician model are structural and could be resolved by making changes to employment model).

146. *See* Andrew P. Winterstein, *The Athletic Trainer-Physician Relationship*, 2 ATHLETIC TRAINING & SPORTS HEALTH CARE 155, 158–59 (2010) (arguing that collaborative relationship between team physicians and athletes "requires attention" and "cannot be taken for granted").

147. *See id.* at 158 (emphasizing that medical professionals need to have open communication and mutual understanding of player-patients); *see also* Calandrillo, *supra* note 130, at 190 (arguing that open relationships between player and team physicians important in minimizing potential conflicts of interest).

148. *See* Keim, *supra* note 62, at 223 (arguing that players and team physicians should work together to "develop a relationship which promotes the athlete's health while simultaneously not unnecessarily excluding the athlete from competition");

will also promote honest communication between the athlete and team physician, allowing the team physician to appropriately diagnose and treat the athlete.[149] Building this collaborative relationship will create trust between the physician and the athletes, resulting in overall better care and treatment.[150]

Moreover, to encourage this open and collaborative relationship between team physicians and players, the NFL should adopt a policy similar to that of the NHL regarding medical disclosure.[151] The NHL's policy is straightforward; it explicitly states what information will be disclosed in certain circumstances and when medical information cannot be disclosed to anyone.[152] By defining situations in which medical information may or may not be disclosed, NFL players will have a better understanding as to when their information may be disclosed to their teams, which will allow for more trust between players and the team physicians.[153]

In addition to encouraging an open relationship with patient-athletes, the NFL should also consider whether the use of the affiliation for medical marketing purposes to bolster the team physician's independent practice is appropriate.[154] In other leagues, these types of relationships are expressly prohibited by each leagues' respective

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see also Deubert et al., *supra* note 91, at 14–15 (arguing that team physicians should "abide by their CBA obligations" and advise players of information disclosed to team). To foster an open relationship, team physicians should take steps to be candid with the players they are treating. *See id.* (arguing that team physicians should clarify role and obligations to players before season, so everyone understands expectations).

149. *See* Deubert et al., *supra* note 91, at 223 (explaining that there should be "shifting burden" in relationship between athlete and team physician). Although athletes should be responsible for "their harm from injuries" during their competitions or games, the team physician has the "initial burden" to correctly diagnose the athlete. *See id.* (explaining that openness and honesty between team physician and athlete allows for best treatment of injury by team physician).

150. *See id.* (explaining benefits of collaborative relationship between team physician and athletes).

151. For further discussion of NHL's medical disclosure policy in the CBA, see *supra* notes 121–126 and accompanying text.

152. *See* NHL & NHLPA CBA, *supra* note 111, at 177–78 (explaining existence of certain situations where team physician must disclose medical information, player must expressly authorize disclosure, and disclosure of medical information is prohibited).

153. *See generally* NFL & NFLPA CBA, *supra* note 43, at 213 (illustrating that CBA does not clearly delineate situations when players' medical information may or may not be disclosed); *see also* Deubert et al., *supra* note 91, at 16–17 (arguing that relationship built on trust starts with open disclosure of expectations between players and team physicians).

154. *See* Dave Goldberg, *NFL's Medical Marketing Partnerships Raise Conflict-of-Interest Issues for Team Physicians*, NFL (Aug. 3, 2007, 9:04 AM), https://www.nfl.com/news/nfl-s-medical-marketing-partnerships-raise-conflict-of-interest-09000d5d801311a6 [https://perma.cc/V685-RD9Q] (explaining NFL does prohibit clubs from entering marketing contracts with health care organizations but does not prohibit health care organizations and physicians from using their team affiliation as marketing tool).

CBA.[155] Medical ethics codes also expressly prohibit conflicts that "could place financial gain ahead of patient welfare."[156] However, no sports leagues' CBA indicates that marketing oneself as a team physician is unethical.[157] This marketing tactic does not take into account the sense of distrust in some of the relationships between team physicians and the athletes, and instead, makes it appear that the athletes "endorse" the team physicians' medical care.[158] As a result, this could substantially increase the team physicians' business outside the team as the general public might only seek treatment from that team physician based upon their status as providing medical care for a certain team.[159] Therefore, teams and leagues should consider putting restrictions on the type of marketing that team physicians may use about their positions to decrease distrust between the team physician and athlete relationship.[160]

Lastly, the NFL should consider whether team physicians should be employed by the National Football League Players Association (NFLPA) instead of individual teams.[161] Allowing the NFLPA to hire team physicians would create a better sense of trust between the player and the physician as the physician's job would be to solely provide care to the athlete, who is the individual benefitting from the NFLPA.[162] This also would allow for a candid physician-patient

155. *See* NBA & NBPA CBA, *supra* note 95, at 380–407 (emphasizing that sponsorship relationships between team physicians and hiring team are expressly prohibited).

156. *See* Eifling, *supra* note 10 (explaining that it is "standard practice" for teams to "sell their affiliations to the highest bidder").

157. *See id.* (explaining situations where team physicians have marketed themselves as providing medical care to specific teams).

158. *See id.* (arguing that team physician marketing implies "players trust [them] with the most vital service they can get in a high-risk occupation," even if players do not trust team physician).

159. *See* Calandrillo, see *supra* note 130, at 192–93 (discussing that marketing agreements "provide significant benefits" in terms of "clients and prestige"). The prestige and increase in clients were demonstrated after Methodist Hospital, who was retained for medical services for the Houston Texans, noticed that new clients were coming to their office solely because of their association with the Texans. *See id.* (citing Bill Pennington, *Sports Turnaround: The Team Doctors Now Pay the Team*, N.Y. TIMES (May 18, 2004), https://www.nytimes.com/2004/05/18/sports/sports-medicine-sports-turnaround-the-team-doctors-now-pay-the-team.html [https://perma.cc/CU54-A9M3]) (explaining that merely being chosen as team physician gives physician right to use affiliation as marketing as "official healthcare provider of a particular professional sports franchise," thus illustrating that there is financial benefit to these relationships even if there is no outward exchange of money).

160. *See* Eifling, *supra* note 10 (implying that team physician marketing may falsely indicate that athletes trust team physicians and endorse their care).

161. For further discussion of the impact of making team physicians employees of the NFL, see *infra* notes 196–223 and accompanying text.

162. *See* Calandrillo, see *supra* note 130, at 195 (arguing that current employment model of teams hiring team physicians perpetuates "profit maximization pressures," prohibiting team physician from "candid relationship" with athlete-patient).

relationship as the physician would not need to consider the best interest of the team and its success during a season.[163] However, a change like this to the employment model of team physicians would have to be formally adopted by both the NFL and NFLPA, and thus, it may be difficult to attain such a major change to a structure that has been utilized by both parties for decades.[164] Additionally, there may be a substantial financial burden placed onto the NFLPA, causing potential concerns regarding the caliber of physicians the NFLPA will choose higher if it now has to foot the bill.[165]

Because of these concerns and the potential difficulty of shifting the financial burden onto other entities such as the NFLPA, all NFL athletes should be permitted to retain and use their own physician for their care.[166] Although the NFL's CBA permits an athlete to obtain a "second opinion" on the care prescribed by the team physician, the athlete should be able to choose a physician for their care, regardless of whether it is a second opinion or not.[167] This would help encourage a sense of trust between the athlete and the physician, as the athlete would not feel forced to use a team physician that is exclusively hired by the team.[168] The NFL or NFLPA could also cover the cost of care or part of the cost so that the athlete could make an informed, *free*, choice to utilize a third-party physician.[169] This would allow the athletes to receive the same top notch medical care, free from conflicting loyalties, in the same manner as when utilizing the team physician.[170]

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163. *See id.* (explaining that current employment model of team physicians does not have "same elements of the [traditional] doctor-patient relationship" as this relationship is "consciously undermined by organization").

164. *See* Swetlitz, *supra* note 139 (suggesting employment structure of NFL team physicians has been present for decades and implementing committee to "make decisions about hiring, firing, and compensation" of team physicians would alleviate current conflicts).

165. *See id.* (observing that changes made to current structure of team physicians could increase cost of care and employment).

166. *See id.* (emphasizing importance of letting athletes choose physician for their care).

167. *See* NFL & NFLPA CBA, *supra* note 43, at 219 (explaining that injured players can choose another physician for second opinion but cannot choose physician for all care). Interestingly, NFL players can choose a "surgeon of their choice" so long as the team physician or any trainers or other medical professionals "will not be responsible for or incur any liability" relating to the surgery. *See id.* at 219–20 (describing only avenue for players to receive medical care besides utilizing team physicians).

168. *See* Calandrillo, see *supra* note 130, at 203–04 (arguing that player would benefit from choosing physician as they would have more confidence in medical care and trust medical advice).

169. *See id.* (arguing that independent physicians chosen by players could be negotiated as part of future terms in CBA to solve dual loyalty problem).

170. *See id.* (suggesting more traditional arrangement would eliminate present conflicts of interest and ultimately result in better care for NFL athletes).

B. The Doctor is Finally Ready to See You: Solutions to Alleviate Implications of Increased Liability on Team Physicians While Protecting Injured NFL Players in the Process

The effects of increasing medical malpractice litigation are felt by everyone involved in the NFL.[171] The growing risk of liability has caused physicians to rethink treating athletes, ultimately affecting the care that athletes receive in the NFL.[172] Although team physicians are financially burdened by the increased liability in medical malpractice litigation, NFL athletes are the ones suffering the physical effects of substandard care and ultimately losing their careers.[173] Moreover, athletes are increasingly skeptical about the care they receive due to their concerns regarding the close connection between the team physicians and their teams.[174] Changes must be made to allow team physicians to properly defend themselves against these lawsuits, but also ensure that players' rights to bring these lawsuits are protected.[175]

1. *Expert Testimony*

Team physicians are advocating for a higher standard regarding expert testimony.[176] The AOSSM has recommended that the requirements for expert testimony should change because sports medicine should be treated as its own specialized field with its own standard of care.[177] The American Medical Association (AMA) has proposed "the minimum statutory requirements for qualification as an expert witness in medical liability issues."[178] In their public statement

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171. For further discussion of the effects of medical malpractice, see *supra* notes 1–10 and accompanying text.

172. *See* Jason Cole, *Lawsuits Are Bad Medicine for Pro Sports Doctors*, WASH. POST (Apr. 26, 2003, 8:00 PM), https://www.washingtonpost.com/archive/sports/2003/04/27/lawsuits-are-bad-medicine-for-pro-sports-doctors/602a51e4-82e5-4c3f-8c6d-a2380097fc0b/ [https://perma.cc/RU9K-V7S3] ("[O]perating on a highly paid athlete is daunting.").

173. For further discussion of effects of medical malpractice on professional athletes, see *supra* notes 1–10 and accompanying text.

174. For further discussion of creating change to preserve the relationship between the team physicians and NFL players, see *supra* notes 141–170 and accompanying text.

175. For further discussion of the importance of change to protect both athletes and team physicians from the costly consequences of medical malpractice litigation, see *infra* notes 244–268 and accompanying text.

176. *See* Am. Orthopaedic Soc'y Press Release, *supra* note 11 (arguing for higher standard for expert testimony as medical malpractice cases involve specialized issues).

177. *See id.* (acknowledging that expert testimony in sports medicine should only be provided by physicians who practice in field).

178. *See Expert Witness Testimony H-265.994*, AM. MED. ASS'N LEGAL MED. POL'Y (2015), https://policysearch.ama-assn.org/policyfinder/detail/medical%20

regarding the increasing liability imposed on team physicians, the AOSSM and supporting organizations stated that testimony regarding any medical care should be elicited from an expert in the corresponding subspeciality.[179] This position is directly aligned with the standard adopted by the AMA .[180]

All jurisdictions recognize different standards for expert testimony in medical malpractice litigation.[181] Some jurisdictions recognize the "same specialty" rule, which requires that an expert witness be a specialist in the defendant-physician's specialty before they can render an expert opinion.[182] Other jurisdictions permit a

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witness?uri=%2FAMADoc%2FHOD.xml-0-1843.xml [https://perma.cc/F9HD-BQNQ] (explaining AMA's proposed statutory requirements for expert witnesses, including parameters for who may qualify as expert witness and encouraging members to serve as impartial expert witnesses if they meet criteria). The AMA believes statutory requirements for expert witnesses should meet the following criteria:

> (i) that the witness be required to have comparable education, training, and occupational experience in the same field as the defendant or specialty expertise in the disease process or procedure performed in the case; (ii) that the occupational experience include active medical practice or teaching experience in the same field as the defendant; (iii) that the active medical practice or teaching experience must have been within five years of the date of the occurrence giving rise to the claim; and (iv) that the witness be certified by a board recognized by the American Board of Medical Specialties or the American Osteopathic Association or by a board with equivalent standards[.]

See id. (indicating that this position on statutory requirements for expert witnesses has been reaffirmed by American Medical Association as of 2021).

179. *See* Am. Orthopaedic Soc'y Press Release, *supra* note 11 (acknowledging that team physician organizations such as NBA Physician Association, NHL Team Physician Society, and MLB Team Physician Association support statement published by AOSSM, including position on higher standard for expert testimony in medical malpractice cases).

180. *See id.* (arguing that requiring expert testimony from expert in their subspeciality would decrease impact of medical malpractice litigation on team physicians). Moreover, it is important to note that the AOSSM believes that team physicians are "subspeciality physicians" because of the multidisciplinary approach of sports medicine. *See id.* (acknowledging that team physicians are "authorities in areas other than sports medicine" and make other decisions regarding "diagnosis, management, [and] surgical treatment" based on expertise in another specialty); *see also Expert Witness Testimony H-265.994*, *supra* note 178 (illustrating that competent expert witnesses would need to have "comparable education, training, and occupational experience in same field as defendant or specialty expertise in disease process or procedure performed in case").

181. *See* Fed. R. Evid. 702 (stating standard for expert testimony in federal cases); *see also* B. Sonny Bal, *The Expert Witness in Medical Malpractice Litigation*, 467 CLINICAL ORTHOPAEDICS & RELATED RSCH. 383, 387–88 (2008) (illustrating several different rules regarding expert testimony in medical malpractice cases and jurisdiction dependent).

182. *See, e.g.*, Pollock v. Panjabi, 781 A.2d 179, 197 (Conn. Super. Ct. 2000) (holding that expert witnesses must provide services to same "degree of care, skill, and proficiency commonly exercised by other members" of profession); *see also* Bal, *supra* note 181, at 384 (explaining that some jurisdictions look to whether standard of care is "degree of care ordinarily exercised by physicians in same specialty under same circumstances").

medical expert witness to opine on matters outside their specialty only if the expert witness is familiar with the alleged negligence.[183] Some other jurisdictions have taken an even stricter approach and require that the medical expert witness have "occupational experience," which is knowledge of the customary practices under similar circumstances, before the expert is permitted to render an expert report or provide any testimony.[184] As a result, the expert testimony standard employed in medical malpractice cases is jurisdiction-dependent.[185] Since medical malpractice cases against team physicians do not have their own body of caselaw, there is no set standard that dictates how to handle these cases.[186]

With these considerations in mind, it would be beneficial for courts to impose a higher expert testimony standard for team physicians, such as adopting the occupational experience rule before qualifying an expert witness.[187] Team physicians are not just experts in their chosen specialties but also have significant training and expertise in sports medicine; therefore, their actions should be evaluated with the occupation experience standard.[188] Specifically, the NFL's CBA states that any team physicians hired after the adoption of the 2020 CBA are required to have a "certification of added qualification in sports medicine" or "certification by a current NFL Head Team Physician that the physician has three years affiliation with an NFL

183. *See, e.g.*, Fitzmaurice v. Flynn, 356 A.2d 887, 892 (Conn. 1975) (holding that medical expert witness was permitted to testify even though they did not practice same specialty as defendant-physician because witness had "requisite knowledge" of standard of practice in that particular specialty).

184. *See, e.g.*, Sinz v. Owens, 205 P.2d 3, 5 (Cal. 1949) (emphasizing that medical expert testimony must be made by expert that has occupational experience, which includes "practical knowledge of what is usually and customarily done by physicians under circumstances similar to those which confronted the defendant charged with malpractice").

185. S*ee* Bal, *supra* note 181, at 384 (explaining expert testimony standards vary depending on jurisdiction and type of injury involved in case).

186. *See* Keim, *supra* note 62, at 202 (explaining that currently there is no caselaw that clarifies what appropriate standard of care is in medical malpractice cases involving professional sports).

187. *See Sinz*, 205 P.2d at 5–6 (observing that it is not fair to admit expert testimony that holds doctor to different standard than what practical experience encourages them to do in situations).

188. *See* NHL & NHLPA CBA, *supra* note 111, at 176–177 (requiring "at least one" physician to be successfully trained in "hockey-specific trauma management training or Advanced Trauma Life Support training"). By requiring its team physicians to have some training in hockey-specific procedures, the NHL's requirements for team physician training make it difficult to show that being a team physician does not require greater training or experience besides what is required in specialty area training and experience. *See Subspecialty Careers: Sports Medicine*, AM. COLL. OF PHYSICIANS (Nov. 2016), https://www.acponline.org/membership/medical-students/acp-impact/archive/november-2016/subspecialty-careers-sports-medicine [https://perma.cc/DGT9-A2S3] (emphasizing that sports medicine requires more specialized training regarding specific athlete procedures and safeguards).

Club's medical staff . . ." and comprehensive knowledge on various NFL health and safety policies.[189] Furthermore, sports medicine has been widely recognized as a "subspecialty" of medicine, indicating the role of a team physician requires more knowledge and experience aside from a physician's chosen medical specialty.[190] Adopting the occupational experience rule for expert testimony recognizes not only the unique role of the team physician as a subspecialist, but also the nature of the employment relationship and the hiring qualification requirements between team physicians and their respective NFL teams.[191]

As a result of this additional expected knowledge team physicians must have, it is imperative that courts recognize that the team physician's role goes beyond their education and training in their subspeciality.[192] By adopting the occupational experience rule, courts will allow evidence to be presented which accurately reflects the wide practical knowledge that team physicians need to know.[193] This will ultimately assist in the determination of whether a breach of the standard of care occurred.[194] Furthermore, adopting this standard will allow courts to shape and create a standardized body of

189. *See* NFL & NFLPA CBA, *supra* note 43, at 213 (explaining additional sports medicine related requirements for team physicians besides board-certified in field of medical expertise).

190. *See, e.g.*, *Sports Medicine Subspecialty Opens New Doors for Emergency Physicians*, AM. COLL. OF EMERGENCY PHYSICIANS (last visited Mar. 15, 2024), https://www.acep.org/life-as-a-physician/careers/sports-medicine-subspecialty-opens-new-doors-for-emergency-physicians#:~:text=Sports%20medicine%20has%20been%20a,his%20entire%20career%20has%20changed [https://perma.cc/AR3P-L6EZ] (implying sports medicine has been recognized as "subspecialty of emergency medicine since 1992, and sports medicine fellowships [are] gradually [finding] their ways into hospitals"); *see also Subspecialty Careers: Sports Medicine*, *supra* note 188 (explaining sports medicine training "requires one year of training beyond general internal medicine residency" and that there is certification program that is "jointly administered by American Board of Internal Medicine, American Board of Emergency Medicine, American Board of Family Medicine, and American Board of Pediatrics").

191. *See Sports Medicine Subspecialty Opens New Doors for Emergency Physicians*, *supra* note 190 (implying that there are additional steps taken to become proficient in sports medicine); *see also* Eifling, *supra* note 10 (arguing that current employment arrangements compromises team physician's perception because of dual loyalty).

192. *See* NBA & NBPA CBA, *supra* note 95, at 380–407 (illustrating team physicians' roles go beyond their specialty knowledge). It is also important to note that team physicians in other organizations are required to complete a fellowship in sports medicine and have additional training in this area. *See id.* (stating that team physician must also have "successfully completed fellowship in sports medicine," "Certification of Added Qualification in sports medicine," or "other sports medicine qualifications as parties may agree").

193. *See Sports Medicine Subspecialty Opens New Doors for Emergency Physicians*, *supra* note 190 (acknowledging that there are additional skill trainings and subject proficiencies that team physician needs to adequately treat athletes).

194. S*ee* Bal, *supra* note 181, at 388–89 (arguing reasonable to elicit expert testimony from expert witness who has practical knowledge and experience to confidently say whether physician breached standard of care).

law that regulates team physicians, which would benefit both physicians and athletes if developed.[195] This is because implementing clear standards will allow courts to make predictable evidentiary decisions regarding expert testimony, emphasizing the importance of recognizing the unique training of team physicians in that their knowledge goes above and beyond their specialty training.[196]

2. *Requiring the NFL to Classify Team Physicians as "Employees"*

Currently, the NFL does not exclusively employ team physicians itself; rather, each individual team is permitted to employ its own team physician.[197] In the past, determinations of whether a team physician is considered an employee or an independent contractor depended on the language of the employment agreement.[198] For example, the NFL's current hiring policy regarding "the structure of the relationship" between teams and their physician depends on the "team's ownership and management," and therefore, every NFL team varies on whether they consider their team physicians to be employees or independent contractors.[199] Since there is no consistent team physician employment scheme as to whether a team physician is an employee or independent contractor, it has been difficult to predict whether a team physician would be individually liable for any medical malpractice claims.[200] As a result, this has created issues with determining whether a player is eligible for compensation under

195. *See* Am. Orthopaedic Soc'y Press Release, *supra* note 11 (arguing that expert testimony should come from physicians who are familiar with multidisciplinary nature of sports medicine to ensure that cases are supported by proper expert testimony).

196. *See* Keim, *supra* note 62, at 202 (implying that beneficial for sports medicine precedent must be developed so there are expectations with medical malpractice claims).

197. For further discussion of hiring requirements for a team physician in the NFL, see *supra* notes 37–50 and accompanying text.

198. *See* Daniels v. Seattle Seahawks, 968 P.2d 883, 887–88 (Wash. Ct. App. 1998) (holding that team physician was team employee, and thus, immune to suit because of employment agreement); *see also* Hendy v. Losse, 819 P.2d 1, 12–13 (Cal. 1991) (holding that since player and team physician were both employed by NFL team, only appropriate remedy was through workers' compensation laws).

199. *See Frequently Asked Questions: Are Team Physicians Employed by the Different Clubs, or Are They Independent Consultants?*, NFL PHYSICIANS' SOC'Y, https://nflps.org/faqs/ [https://web.archive.org/web/20230331040254/https://nflps.org/faqs/are-team-physicians-employed-by-the-different-clubs-or-are-they-independent-consultants/] (last visited Mar. 17, 2024) (explaining that employment structure of team physicians depends on team).

200. *See* Bryant v. Fox, 515 N.E.2d 775, 778 (Ill. App. Ct. 1987) (holding team physician was independent contractor, and thus, plaintiffs' claims were not barred by exclusive remedy provision of Workers' Compensation Act). *But see Daniels*, 968 P.2d at 886–88 (holding team physician was employee, and thus, was immune from individual liability based on lawsuit filed by plaintiff).

the Workers' Compensation Act or whether the player should file a medical malpractice lawsuit, thus creating uncertainty for team physicians.[201]

Although it is apparent through the CBA that the NFL wants physicians who have considerable experience in sports medicine to serve as team physicians, any team physician who is sued by an injured player is typically considered by the courts to be an independent contractor.[202] Despite classifying team physicians as independent contractors, the NFL expects team physicians to harbor knowledge that is NFL-specific to do their job; NFL team physicians are expected to understand and follow league-specific standards and policies like the NFL's concussion protocols and spinal injury protocols.[203] Their role as team physicians for the NFL requires them to be knowledgeable in other areas outside of just sports medicine, such as emergency medicine, so that they can best serve their patient-players.[204] Other professional sports organizations, such as the NBA and NHL, also require their team physicians to have similar specialized knowledge

201. *See* Landis, *supra* note 55, at 153–54 (explaining that several factors are analyzed to determine whether team physician is employee for professional athlete's injury to be covered under Workers' Compensation Act). If a team physician is considered an employee, and thus is covered under the Workers' Compensation Act, then the team physician is immune from state tort claims. *See* Mitten, *supra* note 19, at 213 ("As a co-employee, the team physician is immune from tort liability for improper medical care provided to a player within the physician's scope of employment with the club.").

202. *See, e.g.*, Sherwin v. Indianapolis Colts, Inc., 752 F.Supp. 1172, 1179 (N.D.N.Y. 1990) (indicating that terms of team physicians' agreements with respective NFL team illustrated that they were not "agents or employees" of NFL team, but were independent contractors, and thus, not covered under doctrine of respondeat superior). It is important to note that, as part of their agreement, team physicians are "required to organize and coordinate medical services with a team of referral physicians, dentists and surgeons to treat [NFL] players at all away games . . . obtain additional medical malpractice insurance to cover services rendered to [NFL] players outside [respective state location]." *See id.* at 1180 (implying team physician employment agreement has requirements that go beyond responsibilities of other medical professionals who do not have experience in sports medicine).

203. *See* NFL & NFLPA CBA, *supra* note 43, at 213 (explaining that team physicians must be aware of policies such as "NFL Head, Neck and Spine Committee's Concussion Protocol, the NFL Policy and Program on Substances; the NFL's Behavioral Health Program, the NFL Emergency Action Plan; the prevention, diagnosis and treatment of Heat-Related Illness; and the DICON Program for Infection Prevention in the NFL"); *see also* Becky Sullivan, *Explaining the NFL's Latest Concussion Controversy and Policy Change*, NPR (Oct. 13, 2022, 5:00 AM), https://www.npr.org/2022/10/13/1128524103/nfl-concussion-lates [https://perma.cc/RM4E-SL93] (explaining public criticism of allowing Miami Dolphins' quarterback, Tua Tagovailoa, to return to play after showing obvious concussion signs). Because of the NFL and NFLPA's investigation into this matter, the NFL changed their concussion protocol policy. *See* Sullivan, *supra* note 203 (indicating that all NFL medical professionals, including team physicians and athletic trainers, had to learn new concussion protocol).

204. *See* NFL & NFLPA CBA, *supra* note 43, at 213 (requiring team physicians to have "comprehensive knowledge" of various NFL policies, including concussion protocol).

requirements, but also have additional requirements for knowledge of medicine outside their specialties such as emergency medicine.[205]

To alleviate some of the issues pertaining to the uncertainty of liability surrounding their current employment model, team physicians should be considered employees of the NFL and the NFLPA or the individual teams that hire them.[206] If team physicians were hired as employees of the NFL and NFLPA and then assigned to a team, it would mitigate the sense of loyalty team physicians may feel to the specific team, and in turn, would allow for a more traditional doctor-patient relationship that would focus on the athlete's, rather than the team's needs.[207] Alternatively, if team physicians were to be considered employees of the specific team they work for, they should advocate for their teams to provide a "clear and express employment agreement" that explicitly states, in writing, the duties, responsibilities, and expectations of the physician.[208] This agreement would ensure that the team physician could focus solely on providing proper care to their athletes, without any external influences or pressures.[209] However, the creation of such an agreement, which would set forth that team physicians are employees of the individual teams or the NFL and NFLPA, could impose vicarious liability for the actions of the physician on both the NFL and the individual NFL team that hired the physician.[210] Although imposing vicarious liability on the

205. *See* NHL & NHLPA CBA, *supra* note 111, at 176–77 (requiring "at least one" physician to be trained in "hockey-specific trauma management training or Advanced Trauma Life Support training"); *see also* NBA & NBPA CBA, *supra* note 95, at 380 (requiring team physicians to have "current certification in Basic Life Support, Basic Trauma Life Support, Advanced Cardiac Life Support, or Advanced Trauma Life Support"). The NBA also reserved the right to imposed additional requirements or promulgate any rules based on medical needs of players so that the team physicians may best medically support the players. *See* NBA & NBPA CBA, *supra* note 95, at 380 (explaining NBA can impose additional requirements so players are best supported by team physicians).

206. *See* Nick DiCello, *No Pain, No Gain, No Compensation: Exploiting Professional Athletes Through Substandard Medical Care Administered by Team Physicians*, 49 CLEV. ST. L. REV. 507, 534 (2001) (suggesting benefits of team physicians being hired by NFL and its players union).

207. *See id.* (explaining potential positive change in physician-athlete relationship if NFL and NFLPA employ team physicians).

208. *See id.* at 535 (arguing that establishing "a definite legal standard governing sports medicine" would improve quality of care provided to athletes by team physicians).

209. *See id.* (explaining that clear employment agreements would eliminate present conflicting interests and give complete autonomy back to physicians to make proper medical treatment decisions for their patients).

210. *See id.* at 521 (referring to NFL team's concern that it might be subject to "vicarious liability for malpractice of physician" if court determines team physician is team employee). *But see* NFL & NFLPA CBA, *supra* note 43, at 261–67 (announcing that NFL and teams are subject to Injury Grievance process, and that claims against NFL team and employees are subject to mandatory arbitration under doctrine of respondeat superior).

NFL and hiring team is likely not seen as an incentive to designate team physicians as employees, doing so would motivate the NFL and the hiring team to encourage team physicians to provide the players with the best care possible so that both entities could avoid medical malpractice liability.[211]

Moreover, requiring the NFL to recognize team physicians as employees would enable the league to assume responsibility for a portion of the physicians' medical malpractice premiums.[212] Since 2019, medical malpractice premiums have been steadily increasing, imposing crippling insurance costs on physicians who work high risk jobs where such insurance is deemed a necessity.[213] Although federal law does not require physicians to carry medical malpractice insurance and the insurance premiums often fluctuate between states, and even counties within the same states, at least eighteen states have laws that mandate medical malpractice insurance.[214] Although the NFL may not want to assume the costs of covering its physicians' medical malpractice insurance, it may be in the NFL's best interest to do so to preserve the supply of willing team physicians.[215]

Due to state specific laws regarding malpractice insurance and increasing chances of liability for sports medicine related disciplines, it would be impractical for team physicians to refrain from getting

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211. *See* DiCello, *supra* note 208, at 221–22 (emphasizing that holding NFL and individual teams vicariously liable for actions of team physicians may limit "strong economic motives of business," and encourage team physicians to ensure medical decisions are in best interests of players).

212. *See* Teresa Herbert, *Are Player Injuries Adequately Compensated?*, 7 SPORTS L.J. 243, 267 (2020) (implying that team physicians, "at direction of their insurance companies," advocate for employment contracts so that medical malpractice premiums are covered by NFL team).

213. *See Surge in Medical Liability Premiums Increases Reaches Fourth Year*, AM. MED. ASS'N (Apr. 19, 2023), https://www.ama-assn.org/press-center/press-releases/surge-medical-liability-premiums-increases-reaches-fourth-year [https://perma.cc/H7KS-85AV] (arguing overturning tort reform protections and increasing insurance payouts, not COVID-19 pandemic, contributed to increasing premium costs); *see also Medical Liability Reform Now! 2024*, AM. MED. ASS'N 2 (2024), https://www.ama-assn.org/system/files/mlr-now.pdf [https://perma.cc/K8UQ-AYCT] ("[P]hysicians in certain states and specialties can face liability premiums of over $100,000, and even more than $200,000 per year as is the case for some obstetrician/gynecologists in Florida and Illinois.").

214. *See* Donovan Weger, *Going Bare – Are Doctors Required to Have Malpractice Insurance?*, GALLAGHER HEALTHCARE (Mar. 19, 2021), https://www.gallaghermalpractice.com/blog/post/going-bare-are-doctors-required-to-have-malpractice-insurance/ [https://perma.cc/PTW7-Y5YS] (explaining that federal law and thirty-two states do not require medical malpractice insurance or have any "minimum carrying requirements"); *see also* 49 PA. CODE § 25.281 (1992) (requiring Pennsylvania physicians, hospitals, and other healthcare providers to "maintain the required amount of professional liability insurance or an approved self-insurance plan").

215. *See* Bell, *supra* note 13 ("'A prominent orthopedic surgeon who takes care of professional athletes has already indicated that his insurance will no longer allow him to take care of this population because of this very issue,' Miller said.").

medical malpractice insurance.[216] In recognizing current hesitations about accepting team physician roles from younger physicians, classifying the team physician role as an employee of the NFL would provide the NFL with the ability to absorb a portion of the costs associated with medical malpractice insurance; this would not only make the NFL a highly attractive employer for young physicians, but also help to ensure that team physicians' focus remains solely on the health and wellbeing of the athletes they treat.[217]

Designating team physicians as employees of the NFL would also result in positive changes to the physician-athlete relationship; however, the reclassification of this role may also exacerbate current conflict of interest issues.[218] First, team physicians would be completely managed by the teams and the NFL, and therefore, would have further incentive to prematurely clear athletes to return to play.[219] Second, since team physicians would be directly employed by the teams, it is likely that the physicians would continue to promote their role and affiliation with the team extensively in marketing materials for their independent practices, since they would still be allowed to maintain their private practices.[220] This potential for manipulative over-marketing generates questions as to whether team physicians should even be permitted to reference their roles in marketing materials for their private practices.[221] Despite these

216. *See* Am. Orthopaedic Soc'y Press Release, *supra* note 11 (requiring team physicians to carry "adequate insurance" to protect "the physician, the athlete, and the sponsoring organization").

217. *See* Polsky, supra note 62, at 528 (indicating that team physicians' current medical malpractice coverage may not cover all damages).

218. *See id.* at 524 ("[M]ore conflicts of interest, the more potential exists for doctor to harm patient and to incur liability."). The more the team physician employment model allows more team influence, the more risk players are subjected to when receiving care. *See id.* (implying perception of control could increase if team physicians solely employed by NFL teams with no outside influence).

219. *See id.* at 525 (arguing that concerns may be diminished if team physician is not permitted "from acting as part of team"). For a team physician not to act as part of the team, they would be prohibited from socializing with management, or even the players to make sure they have a non-biased approach to any medical treatment. *See id.* (explaining benefits of encouraging professional relationship between team physicians and team).

220. *See* Keim, *supra* note 62, at 216–17 (arguing employment agreements should not permit team physicians from using position in marketing or allowing team physicians to have financial stake in team's performance); *see also* NBA & NBPA CBA, *supra* note 95, at 380–407 (emphasizing sponsorship relationships between team physician and team is prohibited). To decrease conflict-of-interest issues, the NFL should adopt a similar policy as the NBA to make sure there are no issues between the employment relationship and medical care received. *See* Keim, *supra* note 62, at 216–17 (explaining issues with sponsorship relationships).

221. *See* Keim, *supra* note 62, at 216–17 (arguing that employment agreements should not permit team physicians from using position in marketing or allowing team physicians to have financial stake in team's performance); *see also* NBA & NBPA CBA, *supra* note 95, at 380–407 (emphasizing that sponsorship relationships

conflicts, it still would be preferable for the NFLPA to designate these team physicians as employees so that players' health is adequately protected and remains the biggest priority for team physicians.[222] However, if this is not possible for the NFLPA to provide this type of employment, the NFL should provide safeguards in the CBA to further promote player health and make sure these conflicts are substantially decreased.[223]

3. *Amending the NFL's Collective Bargaining Agreement*

Since both the NFL and NFLPA are involved with protecting player health, amending the CBA would be the easiest way to restructure the role of the team physician.[224] Specifically, the provisions in the CBA that would need to be amended to accomplish this goal are Article 44, which is the injury grievance policy, and Article 39, which governs the players' right to medical care and treatment and defines the role of the team physician.[225] The NFL CBA covers all:

> [P]rofessional football players employed by a member club of the [NFL]; all professional football players who have been previously employed by a member club of the [NFL] who are seeking employment with an NFL Club; all rookie players once they are selected in the current year's NFL College Draft; and all undrafted rookie players once they commence negotiation with an NFL Club concerning employment as a player.[226]

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between team physician and team are prohibited). To decrease conflict of interest issues, the NFL should adopt a similar policy as the NBA to make sure there are no issues between the employment relationship and medical care received. *See* Keim, *supra* note 62, at 216–17 (explaining issues with sponsorship relationships).

222. *See* Keim, *supra* note 62, at 216–17 (emphasizing team physicians' primary role and responsibility should be players' health, without interference from team management or NFL).

223. *See* DiCello, *supra* note 208, at 254–55 (advocating for CBA as remedy to protect player health and make sure role of team physician is clearly defined).

224. *See* Deubert et al., *supra* note 91, at 17–18 (acknowledging CBA is "most straightforward way" to implement changes regarding team physician's role). It is important to note that there is a "long and complicated history" between the parties regarding player health; however, the CBA currently provides many benefits and programs to promote player health. *See id.* (arguing CBA is best avenue to implement changes).

225. For further discussion of proposed changes to NFL CBA, see *infra* notes 231–257 and accompanying text.

226. *See* NFL & NFLPA CBA, *supra* note 43, at xvi (explaining that NFL CBA is NFLPA's "sole and exclusive bargaining representative of present and future employee players in the NFL," formed in accordance with National Labor Relations Act).

This quote makes it obvious that the far-reaching impact of the CBA covers not only the players, but also the NFL as a whole.[227] Considering the uncertainty of the positions between the parties on this topic, there is a concern that amending the CBA to minimize team physician liability could prove unsuccessful as the NFL and NFLPA might have other pressing priorities, such as the use of artificial intelligence, during the next CBA negotiation.[228] Additionally, this option would not be formally considered until after the 2030 season as the current CBA is in place through the 2030 NFL season.[229] It is likely that the implications and risks of team physician liability could be greater by this point in time, making this solution a viable, but less than ideal, option for all parties involved.[230]

4. *Damages Cap for Team Physicians*

Considering the Injury Grievance provision is not applicable to team physicians, the NFL should revise the CBA to quell any concerns team physicians may have about the implications of increased liability in medical malpractice lawsuits.[231] Team physicians are being

227. *See id.* (acknowledging National Football League Management Council as "sole and exclusive bargaining representative of present and future employer member Clubs of National Football League").

228. *See* JC Tretter, *Even in Labor Peace, Players Must #StayReady*, NFL PLAYERS' ASS'N (2020), https://nflpa.com/posts/even-in-labor-peace-players-must-stayready [https://perma.cc/V9NN-X2S6] (explaining that CBA negotiations can be difficult as players have varied interests when negotiating and might not be well-informed); *see also* Shirin Malkani & John Delaney, *AI at Super Bowl Raises Contract, Copyright Issues for Pro Sports*, BLOOMBERG L. (Feb. 9, 2024, 4:30 AM), https://news.bloomberglaw.com/us-law-week/ai-at-super-bowl-raises-contract-copyright-issues-for-pro-sports [https://perma.cc/YPG2-PKA5] (stating that limitations with league's collective bargaining agreement could limit use of artificial intelligence, despite NFL already using it to analyze player health and safety).

229. *See 2020 NFL-NFLPA CBA: Need to Know*, NFL FOOTBALL OPERATIONS (2020), https://operations.nfl.com/inside-football-ops/players-legends/2020-nfl-nflpa-cba-need-to-know/ [https://perma.cc/7ZXG-QSBZ] (indicating 2020 CBA is in place through 2030 season, but includes provisions for improved health and safety with long term implications).

230. *See* Deubert et al., *supra* note 91, at 17 (advocating player health should not "be treated like just another issue for collective bargaining, subject to usual labor-management dynamics"). Instead of formally negotiating this topic and waiting until after the 2030 season, it has been suggested that the NFL should make these policy changes. *See* Deubert et al., *supra* note 91, at 17 (arguing that NFL should take initiative to make changes on own). However, it is not likely this would occur as team physicians have already made their position public and it would not be likely for the NFL to adopt a policy by itself that would be favorable to all parties involved. *See* Herbert, *supra* note 212, at 255 (arguing that state tort claims should be completely excluded from binding arbitration).

231. *See* Herbert, *supra* note 212, at 255 (advocating for amendment to CBA that would allow courts to strictly interpret CBA and maintain integrity of its intended purpose).

required to pay out huge judgments to athletes because of the player's overall substantial increase in pay over the last several years.[232] Thus, the simplest solution to ease team physicians' concerns would be to include them in the Injury Grievance procedure.[233] Adding team physicians into the existing framework would allow players' claims to be arbitrated in the same manner as claims against the NFL.[234] If the NFL adopts this policy, then it is likely that the NFL and NFLPA would also have to revise other sections of its Injury Grievance procedure provision in the CBA to make sure that these parts conform with state tort litigation requirements.[235] However, because this change would affect team physicians who are not NFL employees, it would be difficult to reach a consensus among team physicians to arbitrate these claims as team physicians are not required to join any form of union.[236] It is not likely that the NFLPA would agree to subject *all* state claims to binding arbitration through the Injury Grievance Policy set by the NFL.[237]

Moreover, the NFL should consider advocating for a limitation on damages on behalf of the team physicians.[238] It is important to

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232. *See* Grez, *supra* note 1 (illustrating that team physician was forced to pay $43.5 million verdict in 2023).

233. *See* Eifling, *supra* note 10 (implying that current relationship between team physicians and NFL teams creates conflict of interest that is outside typical freelance contractor scheme, and thus, team physicians should either be employed by NFL teams or players themselves).

234. *See* Deubert et al., *supra* note 91, at 141 ("[R]obust arbitration process is fairest and most efficient way of ensuring that players have same legal rights as regular patients.").

235. *See id.* (acknowledging that "robust arbitration system" should provide players with "roughly comparable remedies to those currently available to them in civil litigation, only now in private and more efficient forum"); *see also* Christopher Deubert, *Aaron Patrick Case Tests Bounds of NFL Collective Bargaining Agreement*, JD SUPRA (May 2, 2023), https://www.jdsupra.com/legalnews/aaron-patrick-case-tests-bounds-of-nfl-9328885/#:~:text=The%20NFL%20and%20Chargers%20removed,and%20the%20NFL%20Players%20Association [https://perma.cc/3RR5-5RX8] (arguing that NFL needs to extend time limitation under Injury Grievance Procedure as limitation could be "fatal" to claims brought).

236. *See* Deubert et al., *supra* note 91, at 141 (arguing for limitation to arbitration mechanism that includes only "NFLPS member doctors" to help create more "cohesive universe of doctors providing care to NFL players and who thus might obtain NFL-specific training or guidance and be subject to code of ethics"). Although it would be preferable to limit the arbitration clause to NFLPS-member doctors, considering that team physicians are considered independent contractors as opposed to employees, imposing a requirement like this would be unduly burdensome and could further alienate physicians from volunteering to be team physicians. *See id.* (emphasizing need to ensure individuals are encouraged to become team physicians).

237. *See* Herbert, *supra* note 212, at 255 (emphasizing that both NFL and NFLPA would have to agree to allow this change and that it is unlikely that they will garner consensus to amend CBA on this concept).

238. *See Damages in Medical Malpractice Lawsuits*, JUSTIA (Oct. 2023), https://www.justia.com/injury/medical-malpractice/damages-in-medical-malpractice-cases/

point out that damages caps are not far-fetched as several states have limitations on damages awarded in malpractice cases.[239] Currently, the CBA already has a salary cap that imposes a limitation on the salary that football players may receive.[240] This salary cap also indicates how this cap may be calculated.[241] The damages cap could follow a similar formula in which the NFL could provide a calculation for earnings damages based upon dividing percentage of the average contract negotiated by respective NFL team by the number of players who have active contracts with that respective NFL team.[242] This damages cap could also reserve the right to pay out medical expenses as the players would easily be able to prove these damages through evidence.[243] With these considerations in mind, it would be best to adopt any type of damages limitations when negotiating the 2030 CBA to make sure favorable calculations for players and team physicians are implemented.[244]

a. The Injury Grievance Procedure

Although the Injury Grievance procedure appears to be beneficial to both sides as it promotes alternative dispute resolution for personal injury and medical malpractice claims, its current language is unclear as to what exactly it covers.[245] Courts have consistently struggled with harmonizing both this provision and

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[https://perma.cc/Y946-3NM4] (explaining damages caps common in medical malpractice cases).

239. *See* Dani Alexis Ryskamp, *The Current State of State Damage Caps*, EXPERT INST. (Apr. 27, 2022), https://www.expertinstitute.com/resources/insights/state-state-damage-caps/ [https://perma.cc/G32K-7EJG] (explaining that twenty-six states cap non-economic damages in medical malpractice cases and six states have caps that limit both economic and non-economic compensation). For example, Nebraska has capped damages for medical malpractice claims that occur after December 31, 2014 at $2,250,000.00. *See* Neb. Rev. Stat. § 44-2825 (2023) (explaining statutory cap on both economic and noneconomic damages).

240. *See generally* NFL & NFLPA CBA, *supra* note 43, at 69–125 (explaining purpose of salary cap and how it is calculated).

241. *See id.* (explaining that salary cap is determined by dividing percentage of revenues negotiated in CBA by thirty-two teams in NFL).

242. *See id.* (explaining purpose of salary cap and how it can increase year to year depending on NFL salaries).

243. *See* Landis, *supra* note 55, at 140 (indicating that proving damages relating to medical expenses are often easier to prove because harm element is typically satisfied by player's injury).

244. *See generally* NFL & NFLPA CBA, *supra* note 43, at 69–125 (explaining that both NFL and NFLPA came to agreement on salary cap calculation so both groups' interests were adequately represented).

245. *See generally* NFL & NFLPA CBA, *supra* note 43, at 261–67 (indicating that Injury Grievance procedure applies to injuries that end employment, but it does not indicate whether procedure bars state tort claims or mandates arbitration of such claims).

the players' right to pursue a state tort claim.[246] NFL teams typically argue that these lawsuits are covered under the scope of the CBA and require arbitration as they should be treated as federal labor law claims.[247] Despite the uncertainty regarding whether the CBA preempts state tort claims, it is important to note that this argument only works when an NFL team is named as a defendant because team physicians that are named defendants are not currently considered employees of the NFL team; instead, team physicians are classified as independent contractors.[248] While the NFL teams named as co-defendants have the luxury of insulating themselves from liability by compelling arbitration, team physicians may still be responsible for paying high judgments without the ability to mitigate the percentage of fault assigned to them in a judgment.[249]

Moreover, even if the Injury Grievance procedure was amended to allow team physicians to be involved in the process, the current language would substantially affect an injured player's rights if this procedure was the only avenue for redress available for a player.[250] The Injury Grievance procedure only allows an injured player to recover their salary for the year they are injured, which substantially decreases an injured player's recovery as it does not allow them to recover additional damages if their time in recovery exceeds a year.[251] Furthermore, courts have broadly interpreted this provi-

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246. *See* Patrick v. NFL, CV 23-1069-DMG, 2023 WL 6162672 at *12–13 (C.D. Cal. Sept. 21, 2023) (holding that negligence and premises liability claims are preempted by NFL's CBA). *But see* Brown v. NFL, 219 F. Supp. 2d 372, 388–90 (S.D.N.Y. 2002) (holding that personal injury case will not be sent to arbitration pursuant to NFL's CBA because claim was based on state law duty, not scope of employment).

247. *See* Sherwin v. Indianapolis Colts, Inc., 752 F.Supp. 1172, 1176–79 (N.D.N.Y. 1990) (holding that plaintiff's claims for negligence, medical malpractice, negligent infliction of emotional distress, and intentional infliction of emotional distress are "substantially dependent" on CBA, and subject to arbitration)

248. *See id.* at 1179 (acknowledging that team physicians are not employees of NFL team, and that claims regarding treatment could "be brought solely against [team physicians], not NFL team").

249. *See* NFL & NFLPA CBA, *supra* note 43, at 261–67 (indicating that procedure only applies to NFL and teams, not team physicians).

250. *See What Is a Grievance?*, NFL PLAYERS' ASS'N, https://nflpa.com/faq/what-is-a-grievance#:~:text=To%20have%20a%20valid%20injury,him%20until%20he%20was%20healthy [https://perma.cc/MKD8-4KLV] (last visited Mar. 16, 2024) (indicating that for player's injury grievance to be valid, player must file "within twenty-five days of when he is released by a club" and can only win "salary for year he is injured, not for any subsequent years"); *see also* NFL & NFLPA CBA, *supra* note 43, at 261–67 (explaining both NFL team and player, share expenses for neutral physician evaluation and arbitration).

251. *See What Is a Grievance?*, *supra* note 250 (explaining remedies under NFL's Injury Grievance Procedure).

sion, and the CBA as a whole, to mandate binding arbitration for any injury claims against the NFL and its teams.[252] As a result, this procedure has subjected claims against the NFL to mandatory arbitration, but has allowed state court claims to proceed against team physicians, creating inconsistent results.[253] Considering the nature of binding arbitration, it is likely the NFLPA would not be amenable to submitting *all* medical malpractice claims against teams and team physicians to binding arbitration as that would completely eliminate the state tort action and any significant remedy.[254] Therefore, the changes to the CBA should focus on decreasing the extent of damages that may be received as part of these claims.[255]

Although these restrictions could be negotiated for as part of the 2030 CBA process, the NFL could also impose new policies to immediately rectify some of the concerns team physicians have about their increasing liability.[256] However, with imposing these policies, the NFL should be mindful to implement policies that do not immediately infringe on an injured player's right to sue for a medical malpractice claim in state court.[257]

IV. DO NOT GIVE UP AT HALFTIME: WHAT IS NEXT FOR NATIONAL FOOTBALL LEAGUE TEAM PHYSICIANS?

A more effective balance needs to be struck between team physicians and athletes, otherwise the NFL risks losing quality medical

252. *See* Smith v. Houston Oilers, 87 F.3d 717, 720 (5th Cir. 1996) (holding that CBA must be strictly enforced and requires mandatory arbitration out of any CBA rooted claim).

253. *See* Sherwin v. Indianapolis Colts, Inc., 752 F. Supp. 1172, 1176–79 (N.D.N.Y. 1990) (holding that state tort claims filed against NFL are subject to binding arbitration; however, claims against team physicians may proceed in state court if they are designated as "independent contractors").

254. *See* Herbert, *supra* note 212, at 255 (arguing that state tort claims must be excluded from arbitration in CBA because allowing such claims to be arbitrated would harm players' chances at winning damages to compensate them for career-ending injuries and allow teams to continue not prioritizing player health).

255. For further discussion of tort reform in CBAs, see *infra* notes 238–243 and accompanying text.

256. *See Updates: 2024 Rules Change Proposals*, NFL FOOTBALL OPERATIONS (Mar. 13, 2024), https://operations.nfl.com/updates/football-ops/2024-rules-change-proposals/#:~:text=During%20upcoming%20league%20meetings%2C%20clubs,2024%20club%20rules%20change%20proposals [https://web.archive.org/web/20240314095616/https://operations.nfl.com/updates/football-ops/2024-rules-change-proposals/] (explaining that rule changes must be "approved by 75% of owners to be adopted").

257. *See* Herbert, *supra* note 212, at 255 (arguing that NFL should be mindful to adopt changes to CBA or rules that do not infringe on NFL player's right to pursue state tort claims in state court).

care for its athletes.[258] High insurance premiums and increased exposure to liability will ultimately decrease the number of physicians willing to specialize in this highly technical and important area of medicine.[259] Professional football is an intensely physical sport, and these athletes deserve exceptional care to support and extend their long-term careers.[260] However, team physicians should also feel comfortable treating these athletes in whatever capacity is required based on the situation.[261] If large monetary awards for medical malpractice lawsuits against team physicians continue in the NFL, it is likely that these physicians will step down from their roles, either by choice or after pressure from their healthcare affiliates or medical malpractice insurance carriers.[262]

Thus, the NFL needs to take a stand to ensure there is no breakdown in the unique relationship between NFL athletes and team physicians.[263] Even if there are multiple solutions to combat this problem, the dire need for immediate change regarding the standard for expert testimony in these high stakes medical malpractice cases and changes to how medical care is treated in the CBA are non-negotiable.[264] By making these changes, the NFL will help physicians feel more comfortable when faced with the risk of medical malpractice lawsuits and the use of expert testimony as evidence at trial.[265]

Although providing some leeway for team physicians to provide patient care in their own way is important to continue to provide the highest quality medical treatment that NFL athletes need, it is also

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258. *See* Am. Orthopaedic Soc'y Press Release, *supra* note 11 (arguing that NFL may lose its core team of sports medicine practitioners if liability of team physicians cannot be limited).

259. *See id.* (emphasizing that likelihood of increased liability and sky-high insurance premiums will discourage physicians from specializing in sports medicine).

260. *See* Keim, *supra* note 62, at 222–23 (stating that it is important to make sure individuals are encouraged to become team physicians so that athletes receive top care).

261. *See* Am. Orthopaedic Soc'y Press Release, *supra* note 11 (explaining that increased liability associated with team physician roles may discourage physicians from accepting such jobs).

262. *See id.* (emphasizing that, without any intervention to prevent heightened impositions of liability on team physicians, insurance carriers and health care employers may encourage these physicians to step down from their roles due to high-risk nature of work).

263. For further discussion of the collaborative relationship between athletes and team physicians, see *supra* notes 145–150 and accompanying text.

264. For further discussion of potential changes to the NFL's CBA and standard for expert testimony, see *supra* notes 176–196 and accompanying text.

265. For further discussion of potential changes to the standard for expert testimony in sports medical malpractice cases, see *supra* notes 176–196 and accompanying text.

paramount to make sure that the players are protected.[266] Thus, the NFL needs to have an open conversation with the NFLPA to ensure that players' views and opinions are represented in discussions about medical care in player-centric documents, such as the CBA.[267] If open conversations are not had, then the NFL risks losing the quality medical care it needs, which would significantly impact the NFL's overall performance.[268]

*Kaitlyn Furst**

266. *See* Mitten, *supra* note 19, at 214 (acknowledging that requirements and rules for team physicians do not benefit athletes, specifically with workers' compensation laws).

267. For further discussion of potential changes to the NFL's CBA, see *supra* notes 224–242 and accompanying text.

268. *See* Am. Orthopaedic Soc'y Press Release, *supra* note 11 (explaining that without solutions to curtail impacts of increased team physician liability, quality and availability of care for professional athletes will drastically change).

* J.D. Candidate, May 2025, Villanova University Charles Widger School of Law; Dual Degree B.A., Music & Sociology, *summa cum laude*, Honors in Music, Moravian University, 2022. This Comment is dedicated to my family and friends who have supported me throughout this journey and during my time in law school. To my parents, Stephen Furst and Carolyn Frisoli Furst, I would not be who I am today without you both. I am especially thankful for all the love and support you have given me throughout the last three years. To my friends, thank you for always being there for me and keeping me grounded during this process. Additionally, thank you to the members of the *Jeffrey S. Moorad Sports Law Journal* for their ongoing assistance throughout the publication process. I also dedicate this Comment to my feline friend, Nica, who supported me in her own way during the long hours working on this piece.